



Health Claims Forum

Neurodiversity and Mental Health

12 November 2024

The Royal College of General Practitioners,
30 Euston Square, London, NW1 2FB

What do we mean by neurodiversity?

“Very simply, neurodiversity refers to the different ways a person's brain processes information.”

Cambridge University Hospitals



What's normal?

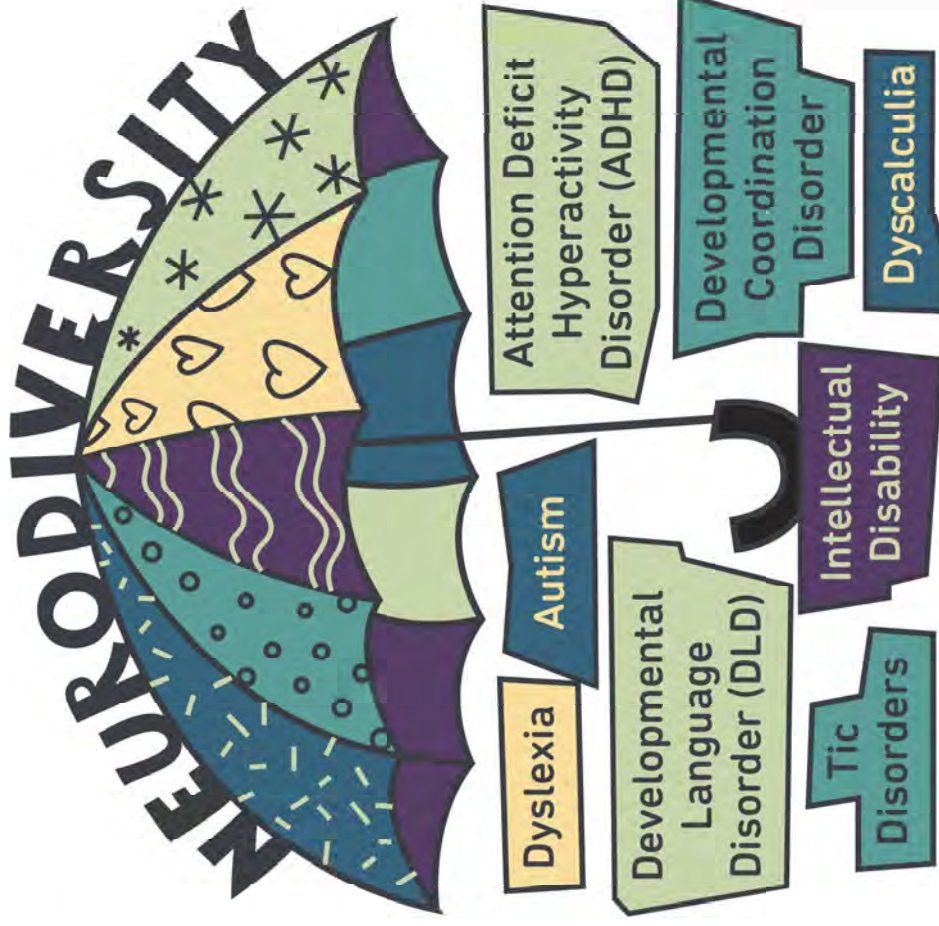
- An individual who thinks, perceives, and behaves in ways that are considered the 'norm' by the general population
- Most institutions and places of employment are typically designed to accommodate people who fit into these norms
- Commonly referred to as “neurotypical”



What is neurodiverse or neurodivergent?

- Whilst everyone's brain develops similarly, no two function alike...each one is unique
- Being neurodivergent means having a brain that works differently from the average, or “neurotypical”, person...
- This results in behaviours and responses that differ from what might be expected socially, physically, or verbally





The scale of the problem...

- It's estimated around 1 in 7 people in the UK have some kind of neuro difference (Deloitte estimate 15-20% or 13 million people)
- The number is increasing, and this may be because there is less stigma, more recognised in schools, or there is a population increase...



The challenges for neurodivergent individuals...

- **Underdiagnosis and Misdiagnosis:**
Many are not diagnosed or are misdiagnosed, leading to a lack of appropriate support
- **Workplace Inclusion:**
Significant barriers in the workplace, including stigma and a lack of accommodations, leaving some unsupported and without proper adjustments
- **Mental Health:**
At higher risk for mental health issues due to societal barriers and lack of support



Prof Christoffel Grobler

How is neurodiversity changing? The background, and the future...

Dr Jonathan Carnell

The challenges for managing mental illness, and neurodiversity, in primary care...

Jo Throp

‘Now you see me – the power of curiosity’

Josh Wellman

Neurodiversity, Mental Health and Workplace Challenges
– A Psychological Perspective



WHAT DO ATTENDEES SAY ABOUT LUCID?

'This was my first LUCID event, and I'm absolutely determined to ensure it's not my last. I thought it was fantastic from registration to close; it was seamless.'

'Wonderful conference and atmosphere. Well done and look forward to 2025!'

'I loved the detailed presentations, the clear and efficiently run programme, the detail in the app and the chance to meet and chat with others.'

'I loved the diversity of subjects...it's all great education to help us when dealing with our customers and when designing new products.'

The LUCID logo consists of the word "LUCID" in a bold, white, sans-serif font, centered within a solid dark red rectangular background.

LIFE UNDERWRITING, CLAIMS AND INSURANCE DOCTORS

A collaborative conference organised by representatives from all the UK & Ireland's industry clubs

The graphic features a dark background with a pattern of glowing red, circular, scale-like shapes. In the upper right, there is a depiction of a red, cloudy sky. Overlaid on this is a white rectangular box containing the event details.

LUCID
BIRMINGHAM, UK
19 - 20 May 2025

THE LEADING INDUSTRY CONFERENCE IN
THE UK & IRELAND FOR LIFE UNDERWRITING,
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The challenges for managing mental illness and neurodiversity in primary care....

Dr Jonathan Carnell

November 2024

INTRODUCTION

A day in the life...

- | | |
|---------------|--|
| • 08:00-08:30 | Check results, referral updates, script queries. |
| • 08:30-12:00 | Morning clinic – 14 booked appointments followed by urgent requests and then up to 6 phone consultations. |
| • 12:00-13:00 | Letters, referrals and other admin from morning clinic, sign prescriptions and deal with medication queries. |
| • 13:00-14:00 | Home visits |
| • 14:00-15:00 | Meetings (Palliative Care, Cluster, Practice Business including completing insurance reports) |
| • 15:00-18:00 | Afternoon clinic – 14 booked appointments followed by up to 6 phone consultations. |
| • 18:00-19:00 | Admin from afternoon including referrals, queries. |

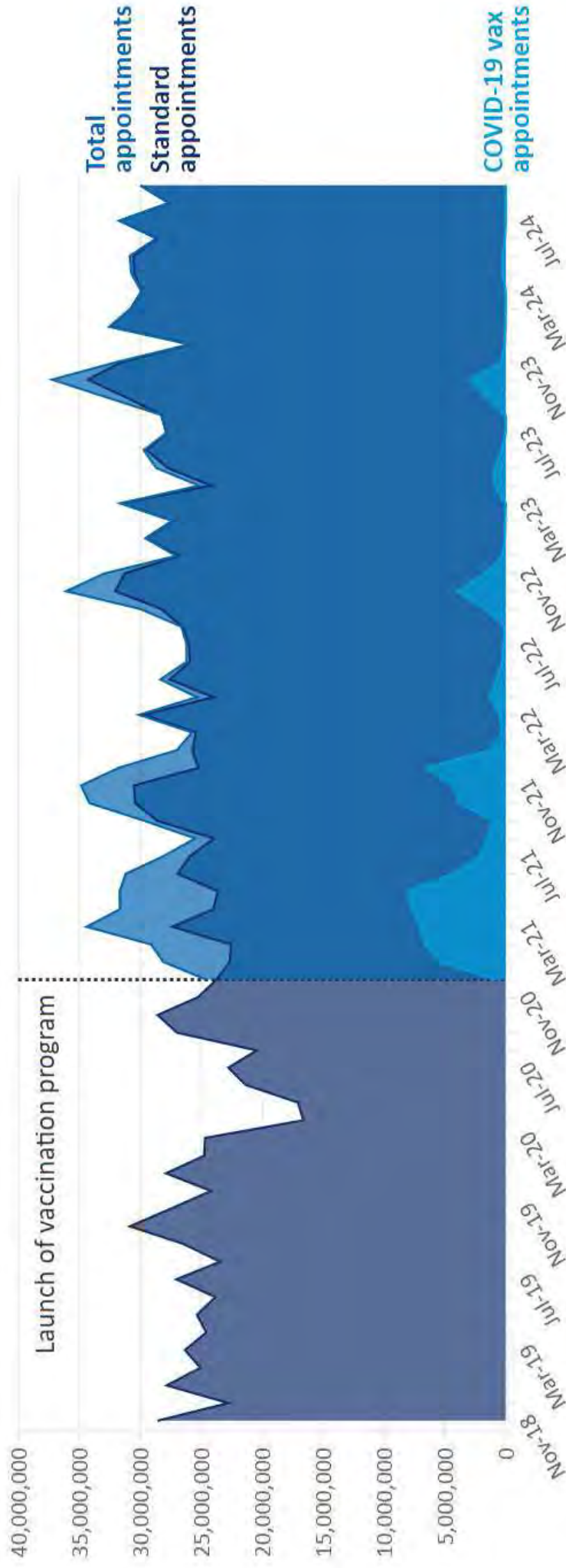
23rd March 2020

“Stay at home, protect our NHS and save
lives”

WHERE ARE WE NOW?

Appointments in General Practice including COVID-19 vaccinations

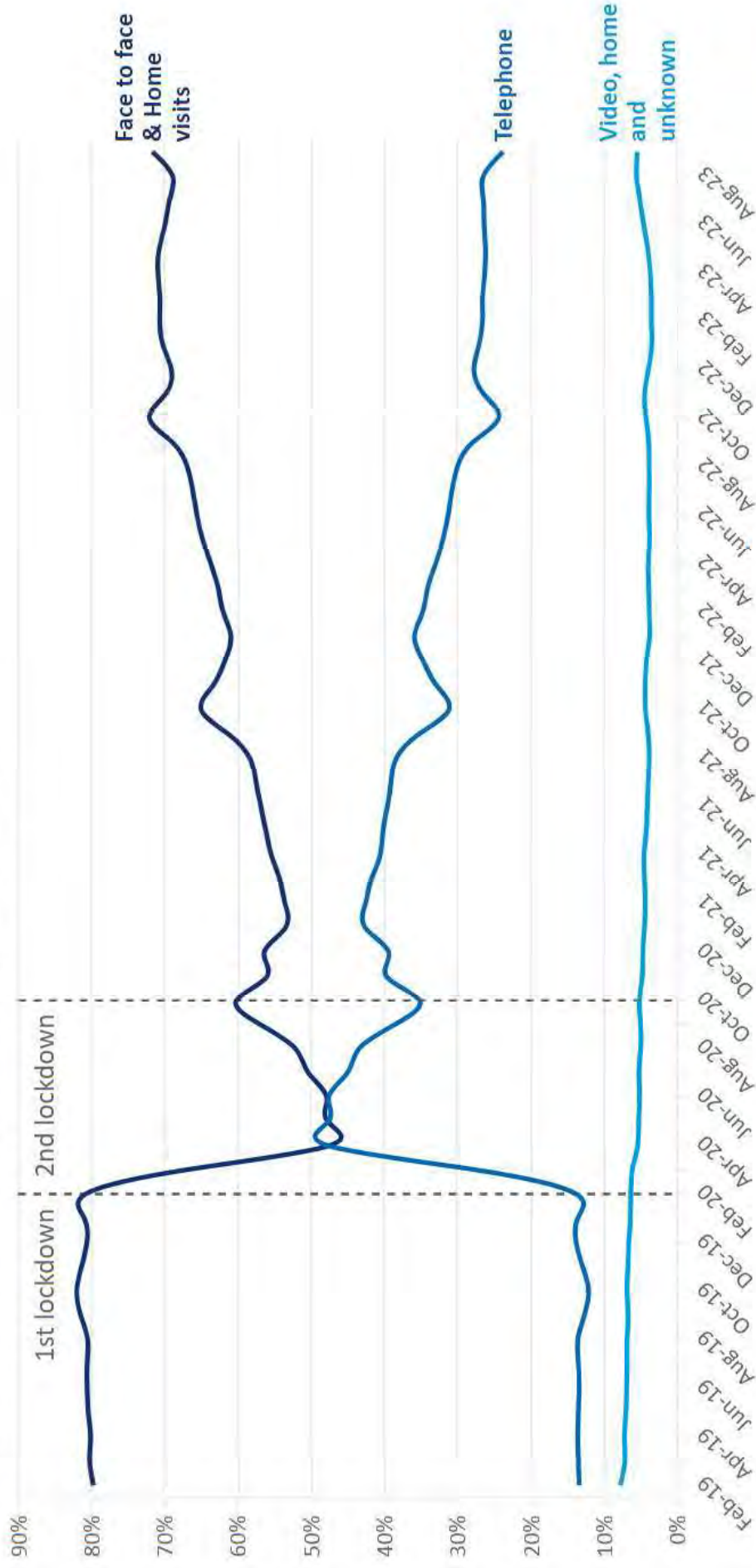
November 2018 to September 2024



Source: Appointments in General Practice • Total count of appointments refers to 'Estimated England total count of appointments' the dataset in which some estimates have been provided to account for missing data.

Appointment formats in General Practice

February 2019 to September 2023, shown as a percentage of total



Source: Appointments in General Practice • Total count of appointments refers to 'Estimated England total count of appointments' the dataset in which some estimates have been provided to account for missing data.

Estimated total number of appointments in general practice by year (2019 to 2023)

■ Estimated appointments (excluding COVID vaccinations) ■ COVID vaccination appointments

Estimated number of appointments

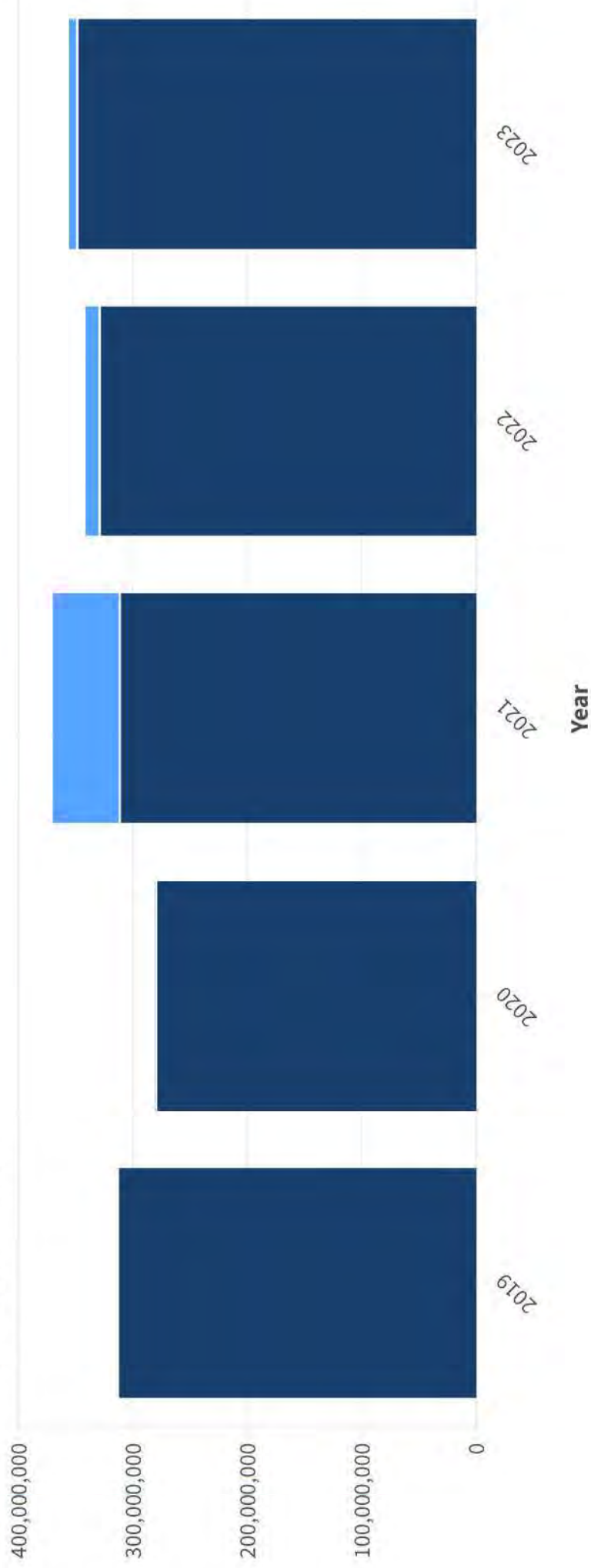


Figure 1: General practice staff delivered 356 million appointments in 2023, which is 14% more than in 2019 [1].

Numbers of all and fully qualified FTE GPs (September 2015 to April 2024)

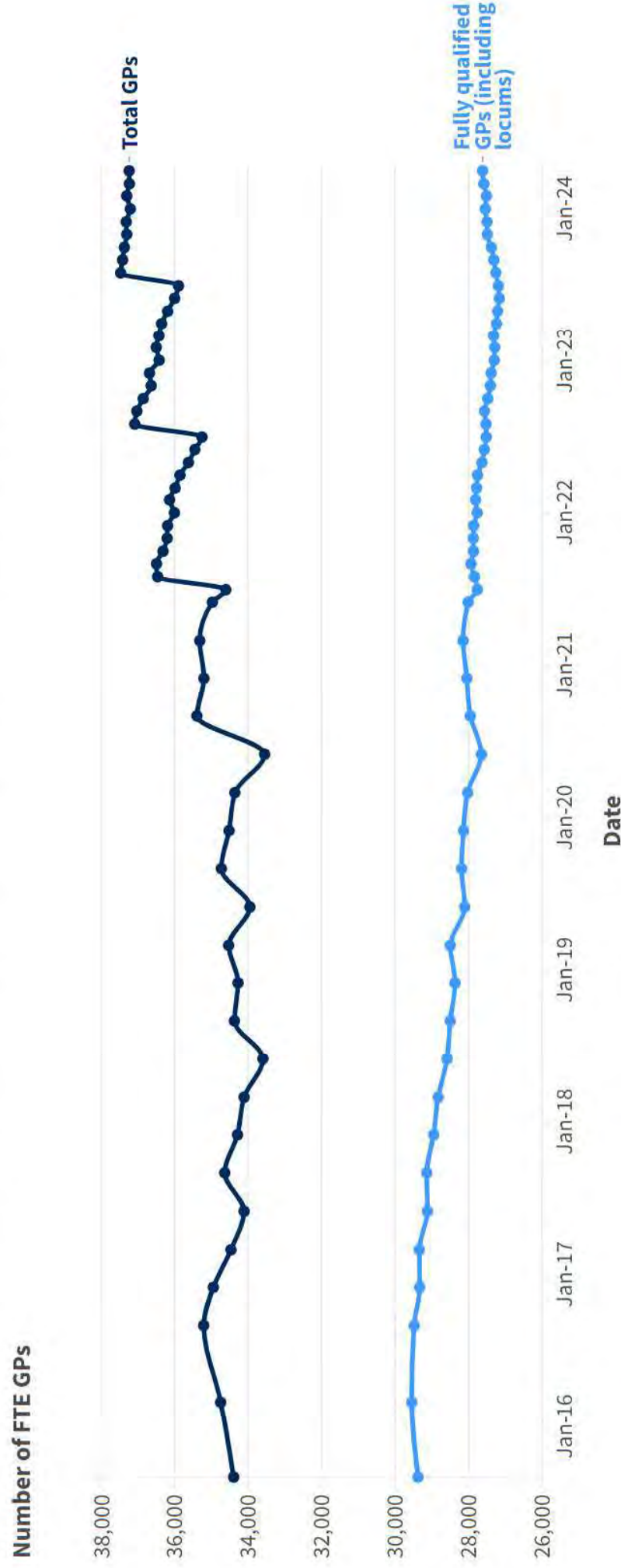
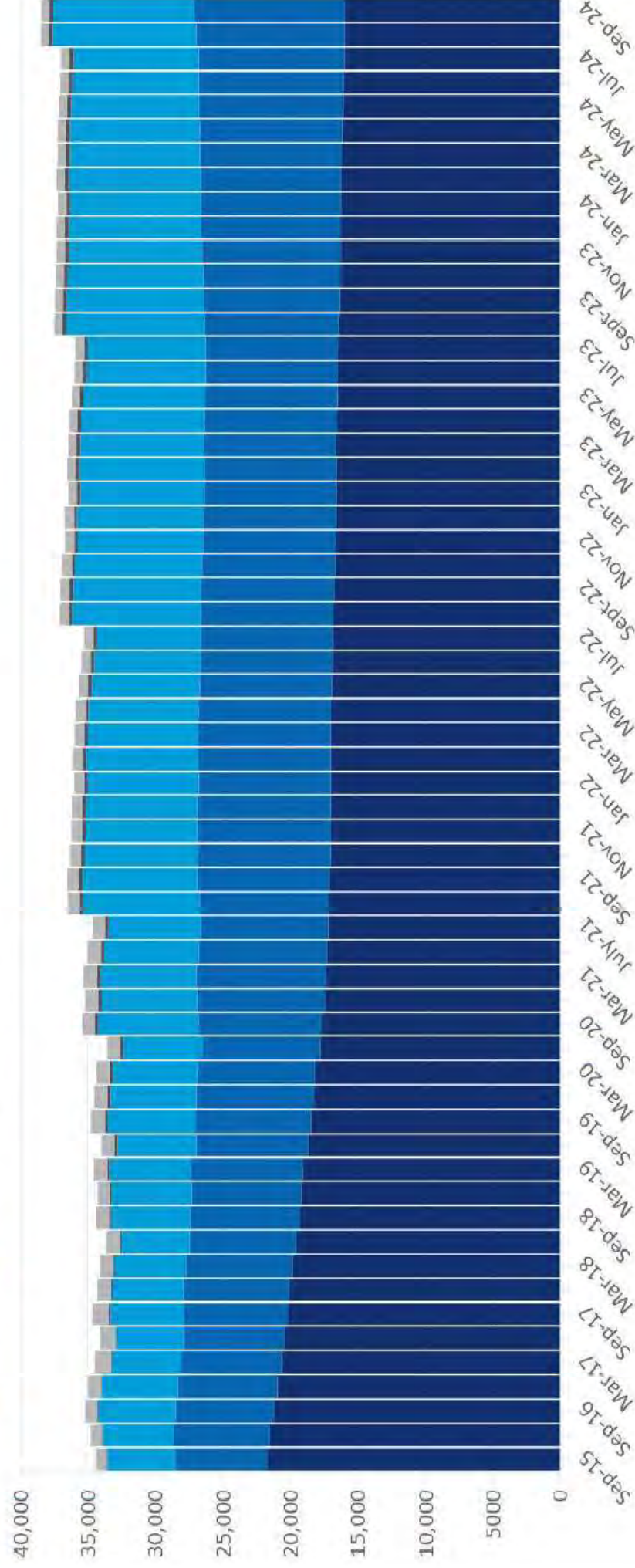


Figure 3: Whilst the total number of GPs in general practice has been increasing due to increasing numbers of trainees, the number of fully qualified GPs (including locums) has been steadily declining. As of April 2024, there are 27,606 fully qualified full time equivalent GPs, 1.9% less than in December 2019. As of April 2024, there are 37,237 full time equivalent GPs (including trainees), 7.9% more than in December 2019 [2].

Number of NHS GPs by role (FTE) - all GPs including trainees

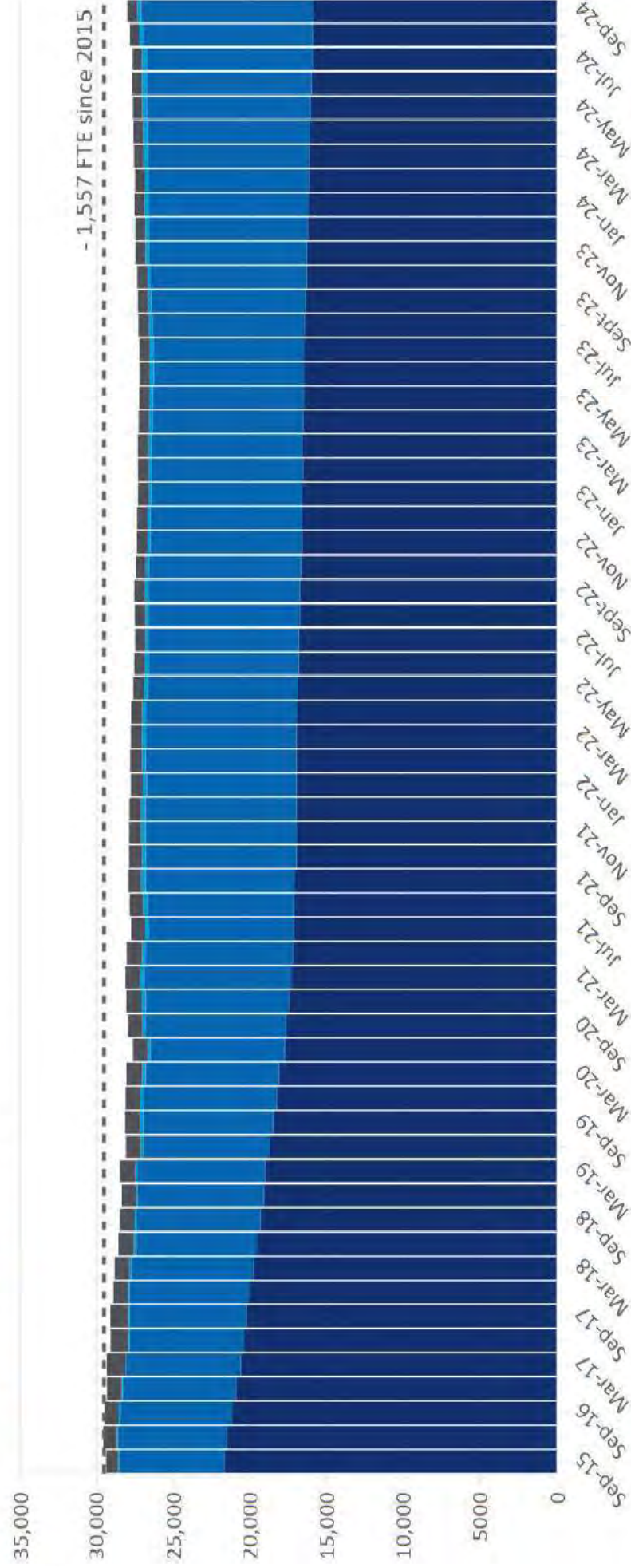
September 2015 to September 2024

Partners Salaried Trainees Retainers Locums



Number of NHS GPs by role (FTE) - fully qualified GPs only

September 2015 to September 2024



Source: [NHS Digital General Practice Workforce Statistics](#)

Number of GP Partners, and Salaried and Locum GPs (Headcount)

September 2015 to September 2024



Source: General Practice Workforce Statistics • Salaried includes GP retainers.



Work commitment of qualified permanent GPs per week (headcount)

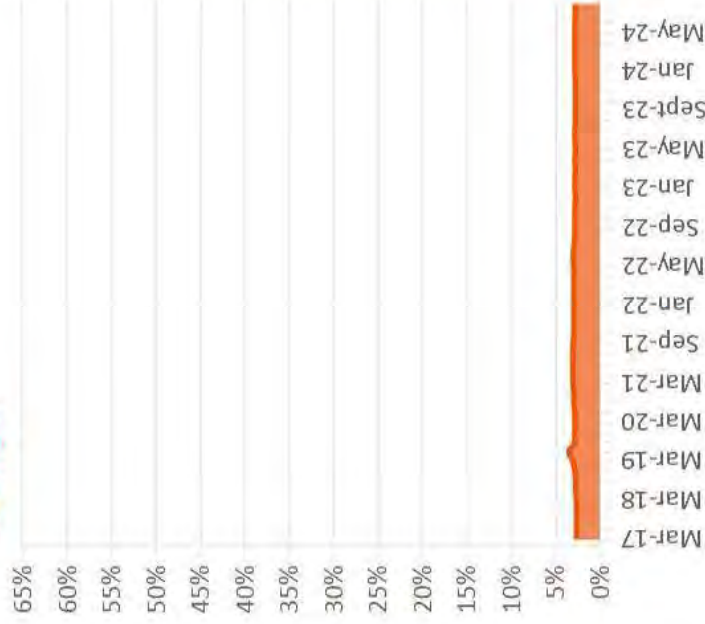
March 2017 to September 2024

Shown proportionally. Excludes GP trainees and locums.

GP Partners



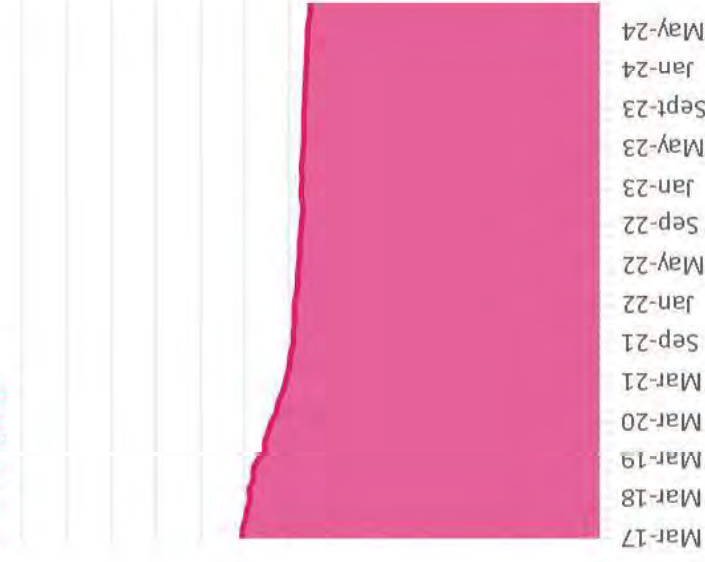
< 15 hours



15 to 37.5 hours



> 37.5 hours



Source: General Practice Workforce Statistics



Numbers of nurses and direct patient care staff (September 2015 to April 2024)

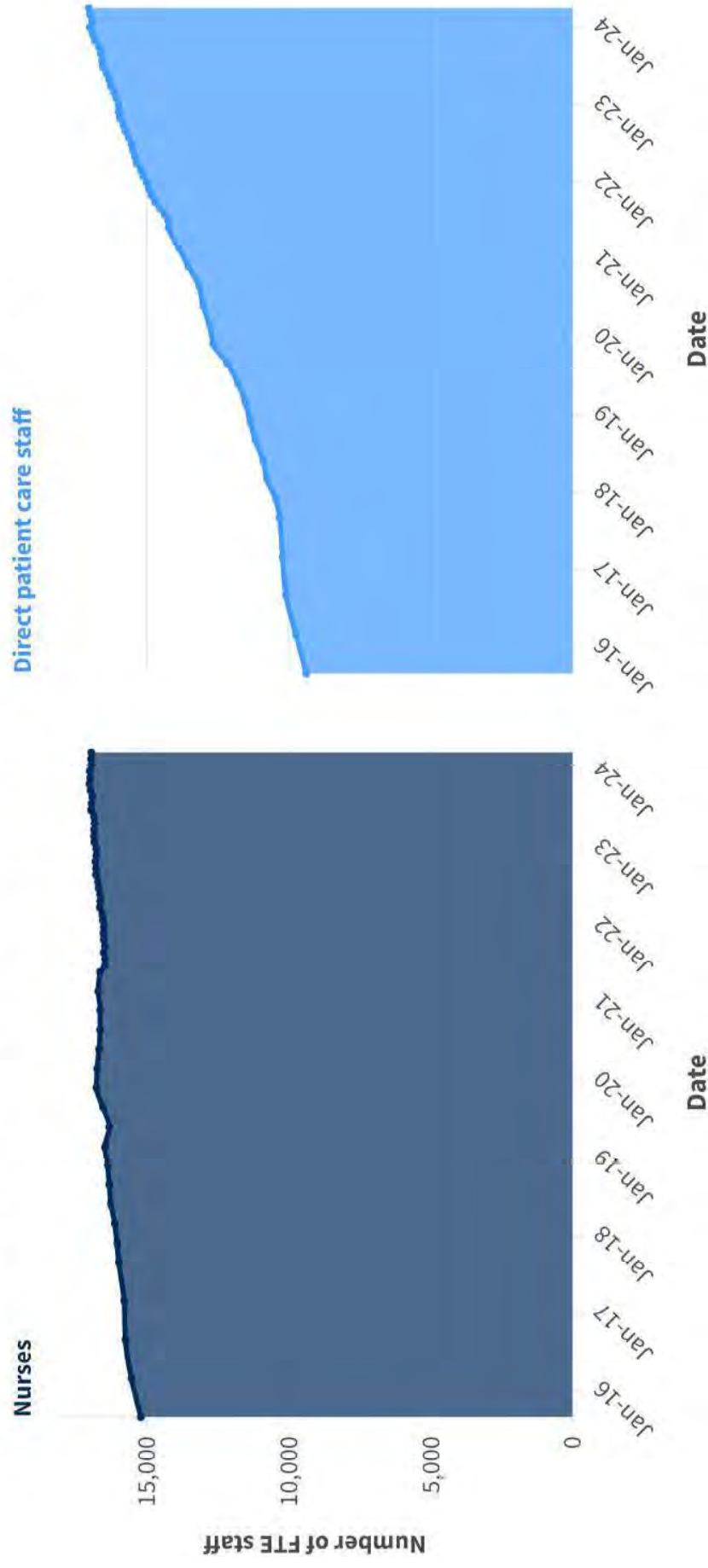
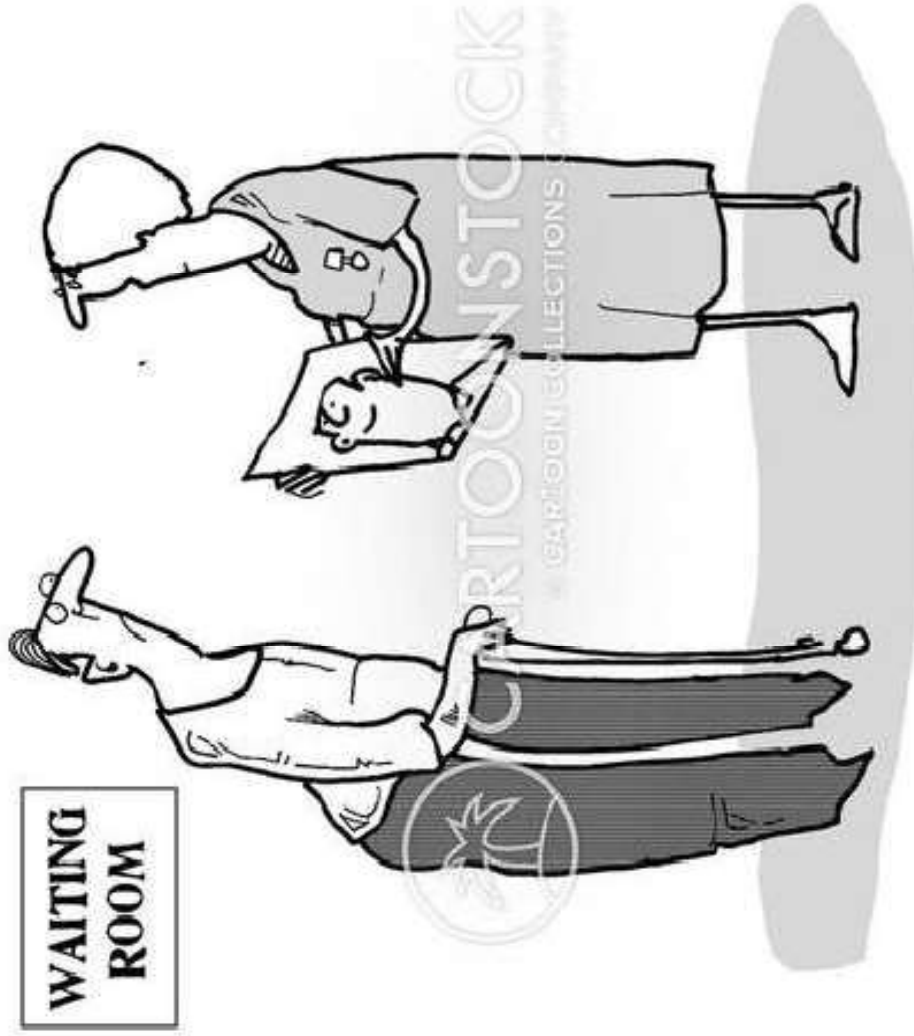


Figure 4: The number of nurses and direct patient care staff have increased since 2015. There are 16,946 FTE nurses working in general practice as of April 2024, which is 1.1% more than in December 2019. There are 17,031 FTE direct patient care staff working in general practice as of April 2024, which is a 35% increase from December 2019 [2].

Number of patients per FTE fully qualified GP (September 2015 to April 2024)



Figure 2: The average number of patients per GP has seen a steady increase since 2015. There are 2,294 patients per GP as of April 2024, which is an increase of 7.2% since 2019 [2].



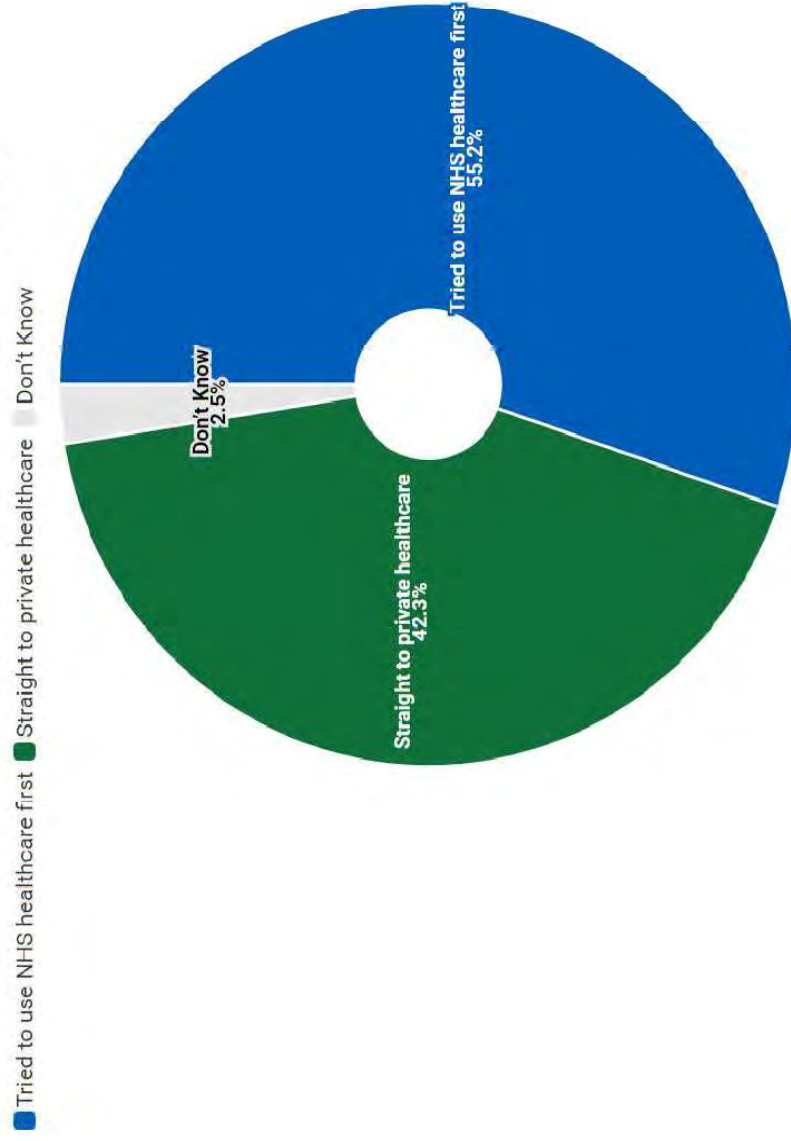
Labour have promised everyone can see their GP within
48 hours...here's yours!

“GOING PRIVATE 2024” SURVEY

- Almost seven in ten of people (67%) say that they would consider using private healthcare
- Almost one third (32%) of people surveyed stating they had previously used private healthcare.
- Two in five (42%) went straight to pay for care rather than trying the NHS first.
- Of those that have used private healthcare, six in ten (58%) said they paid through PMI compared with under half (49%) in 2023.
- 55% of respondents indicated that they would be somewhat or much more likely to apply for a job if it came with Private Medical Insurance.
- Vast majority (88%) of people who have used private healthcare considered it worth the expense.

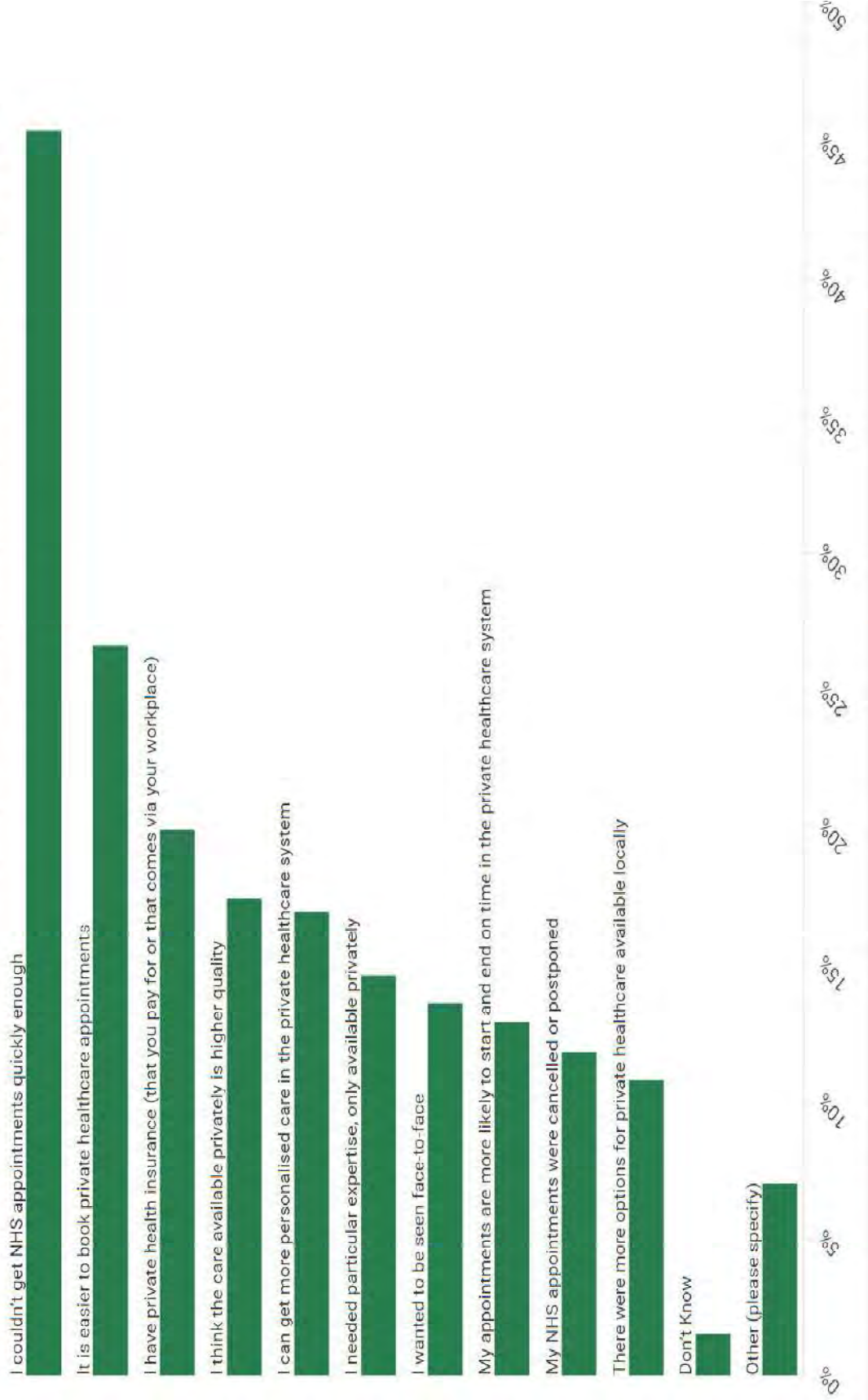
Straight to private healthcare or trying the NHS first?

Did you try to use the NHS first, or did you turn immediately to private healthcare?



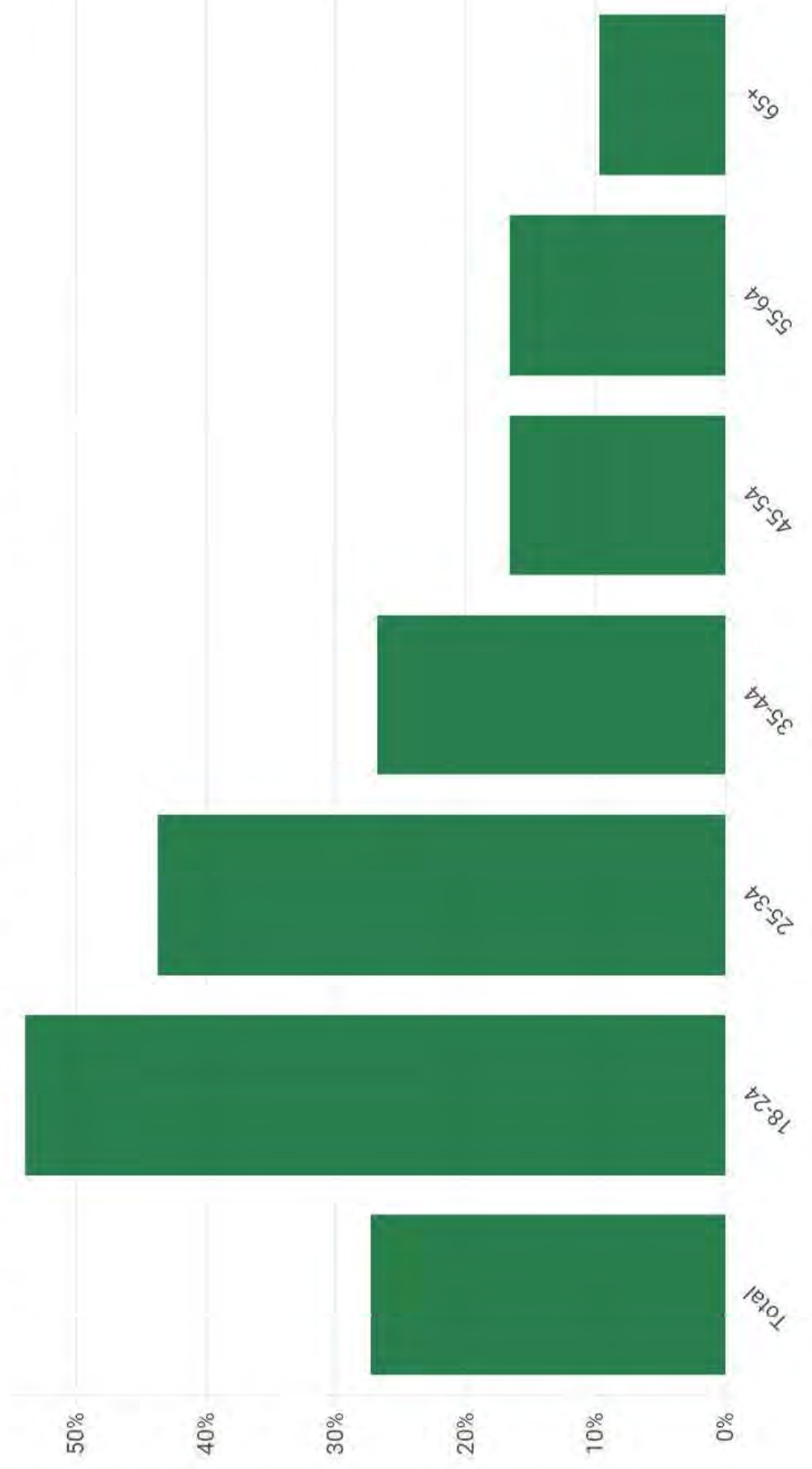
Reasons for choosing private healthcare

Which of the following reasons best explains why you have used private healthcare? Please select all that apply



Younger people are more likely to have accessed private GP services

Percentage of people who have had a private GP appointment who have used private healthcare



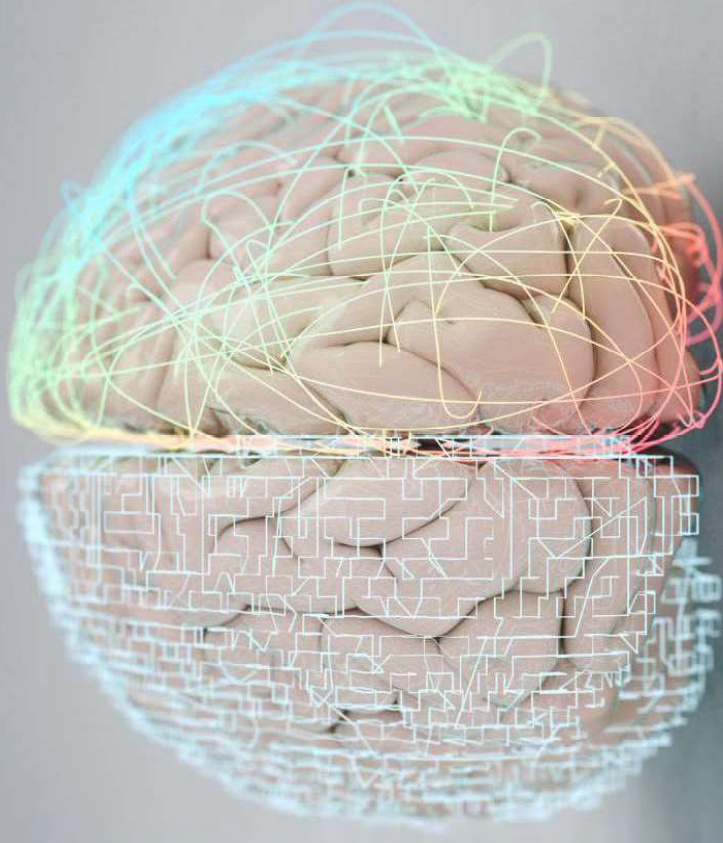
Younger people are much more likely to self-pay than older people



NHS vs Private Consultations

- Limited data on exact numbers
- NHS GP should be included in correspondence
- Prescriptions – interactions / side effects
- Virtual vs Face to Face
- Issues with follow up and “Shared Care”

lth
sit



Around 40% of all GP appointments are related to mental health problems

Between 15-20% of the UK population
thought to be neurodiverse, with
significant underdiagnosis in adults

“Estimated up to 500,000 aged 20-49 years and
up to 600,000 aged 50+ with autism may be
undiagnosed.”

O’Nions et al, Lancet, 2023

Neurodiversity and Mental Health overlap

- Overlap within neurodiverse conditions (e.g. ADHD / Autism)
- Increased rates of anxiety and depression
- Epilepsy
- Learning Disability

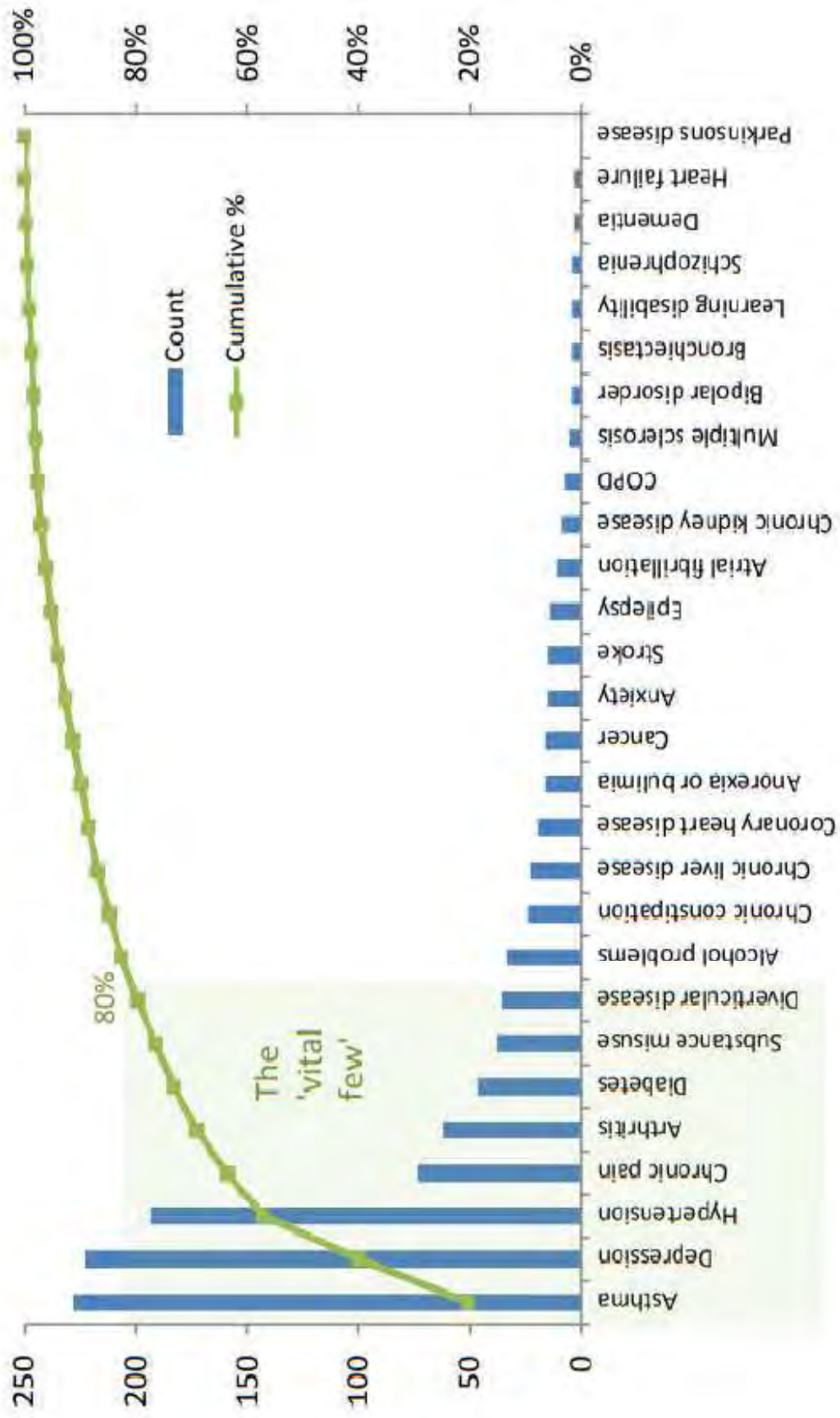
- GP
 - School / SENCO
 - NHS 111-2
 - A+E
 - Primary Mental Health Team
 - Community Paediatrics
 - Secondary Care (CAMHS)
 - Psychiatric Inpatient admission
-
- Private Sector Assessments

- GP
- NHS 111-2
- Mental Health Practitioner (MHP)
- Primary Care Mental Health Service (PMHSS)
- Secondary Care Outpatients (CMHT)
- Psychiatric Inpatient admission
- Private Sector Assessments

Neurodiversity assessment and diagnosis

- Children / Adolescents vs Adults
- School
- Increased awareness
- Online assessments
- Self referral

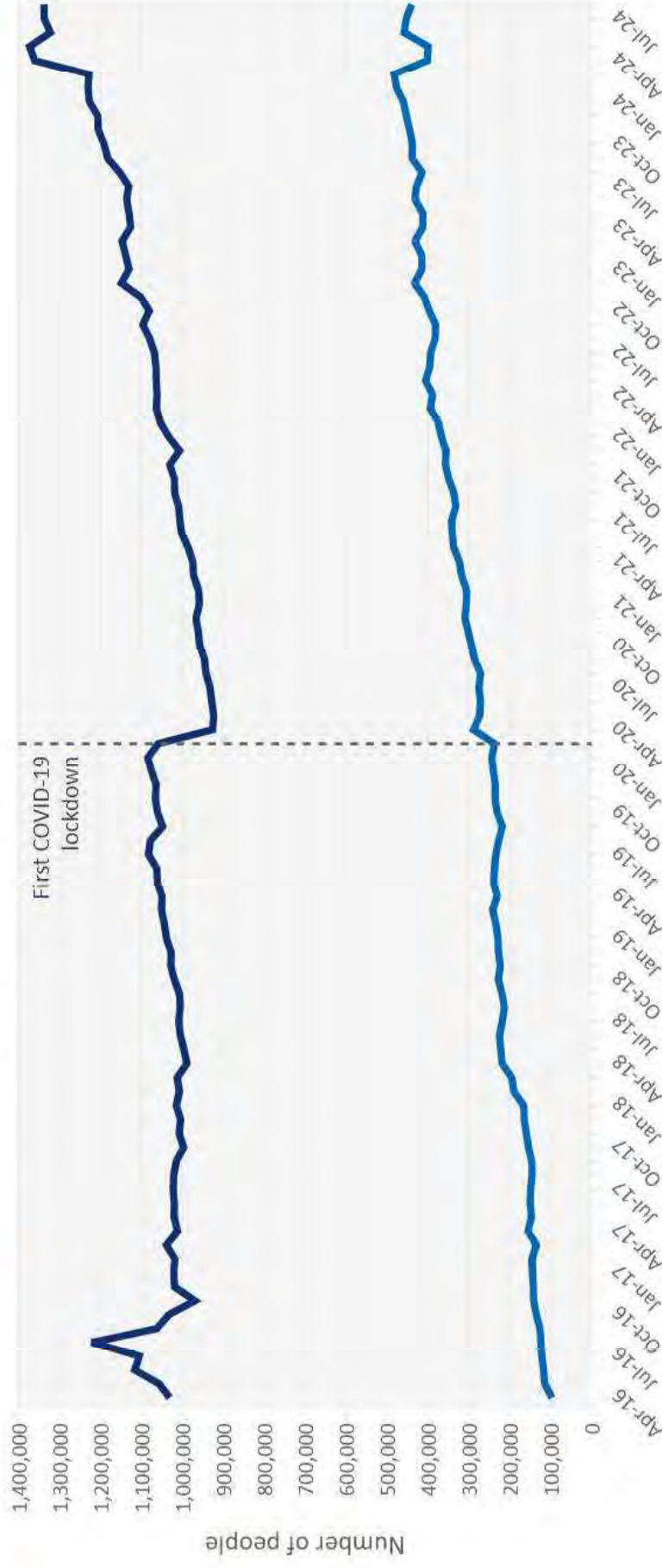
Pareto - disease readcodes April 2020



The number of people in contact with mental health services

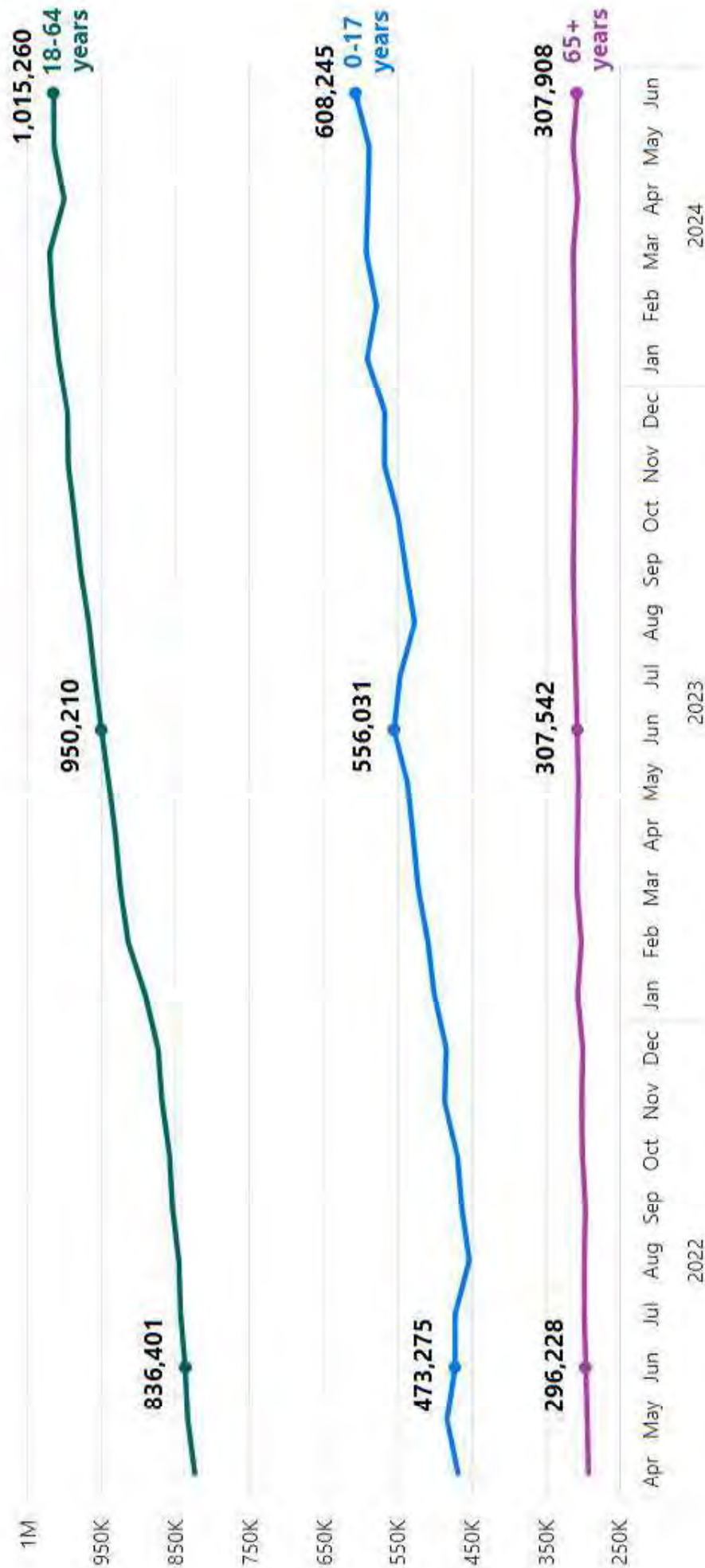
April 2016 - August 2024

■ Adult mental health services ■ Children and young people's mental health services

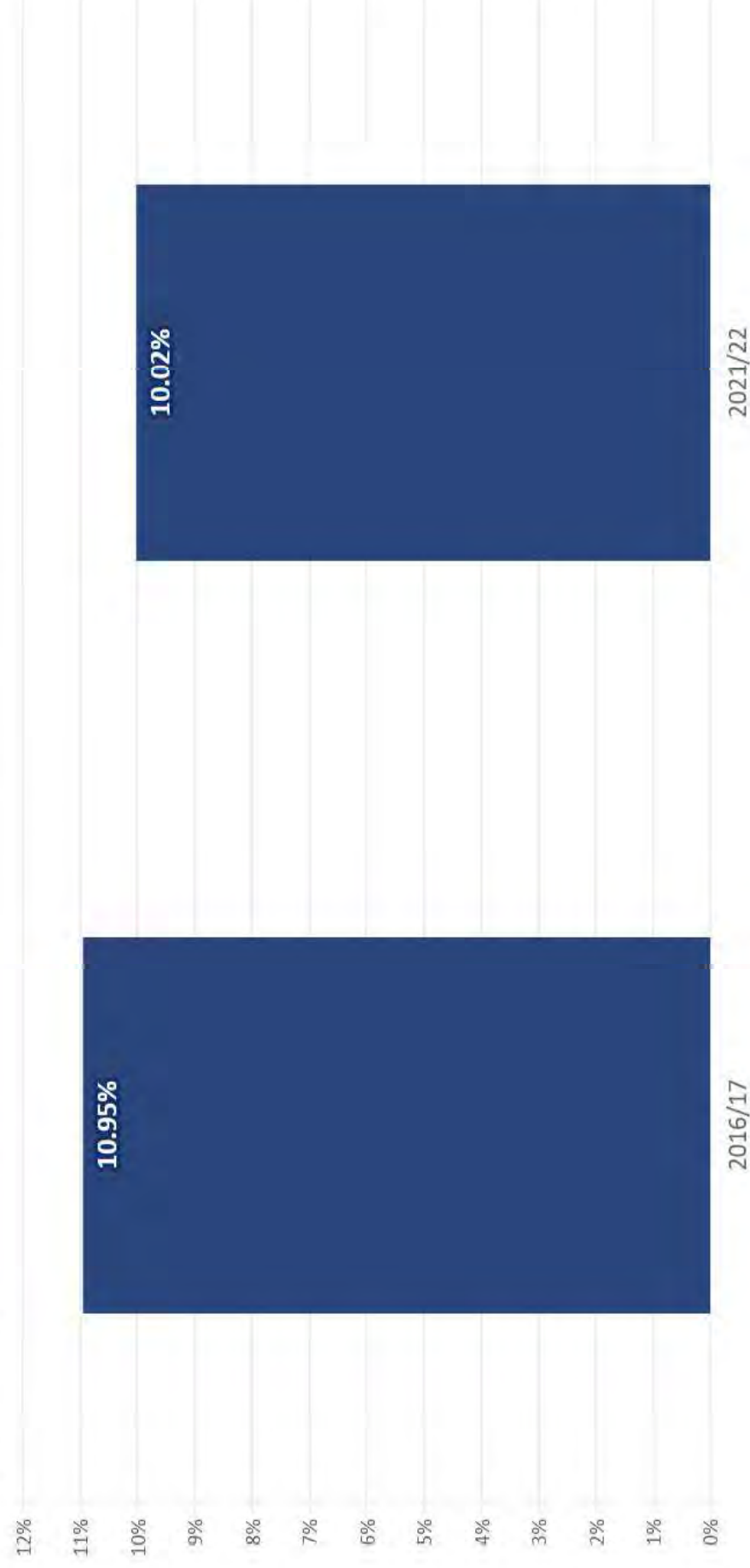


Source: NHS Digital Mental Health Services Monthly Statistics • Data shows the number of people in contact with adult mental health services (AMH01) and children and young people's mental health services (CYP01) at the end of the month. Due to a cyber incident affecting a number of mental health providers, data between August 2022 and March 2023 is estimated by NHS England.

Figure 7: People in contact with mental health services by age group, April 2022 to June 2024



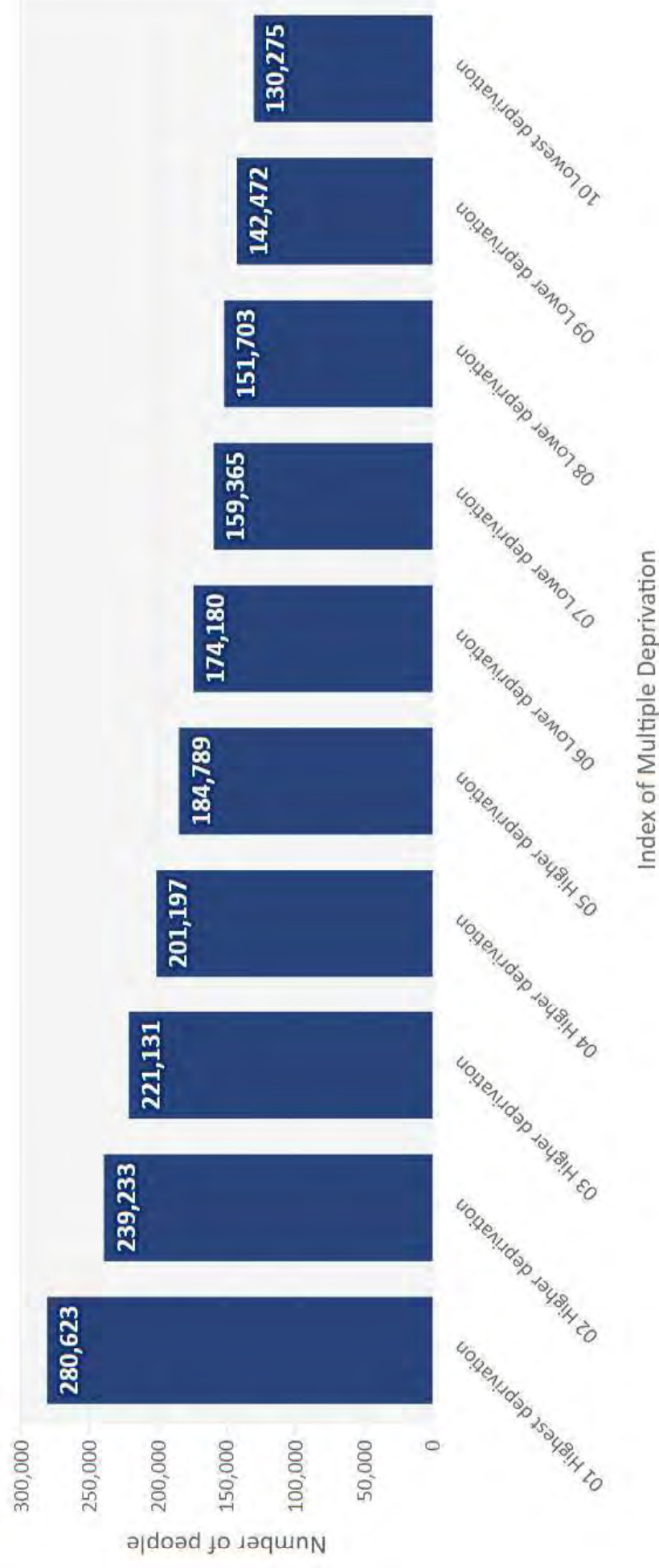
Mental health spend as proportion of total NHS England spend in real terms 2016/17 - 2021/22



Source: BMA analysis of NHS England mental health expenditure, NHS England total expenditure • Real-terms analysis (2021/22 prices) based on GDP deflators at market prices June 2022 (Quarterly National Accounts)

The number of people in contact with mental health services by Index of Multiple Deprivation

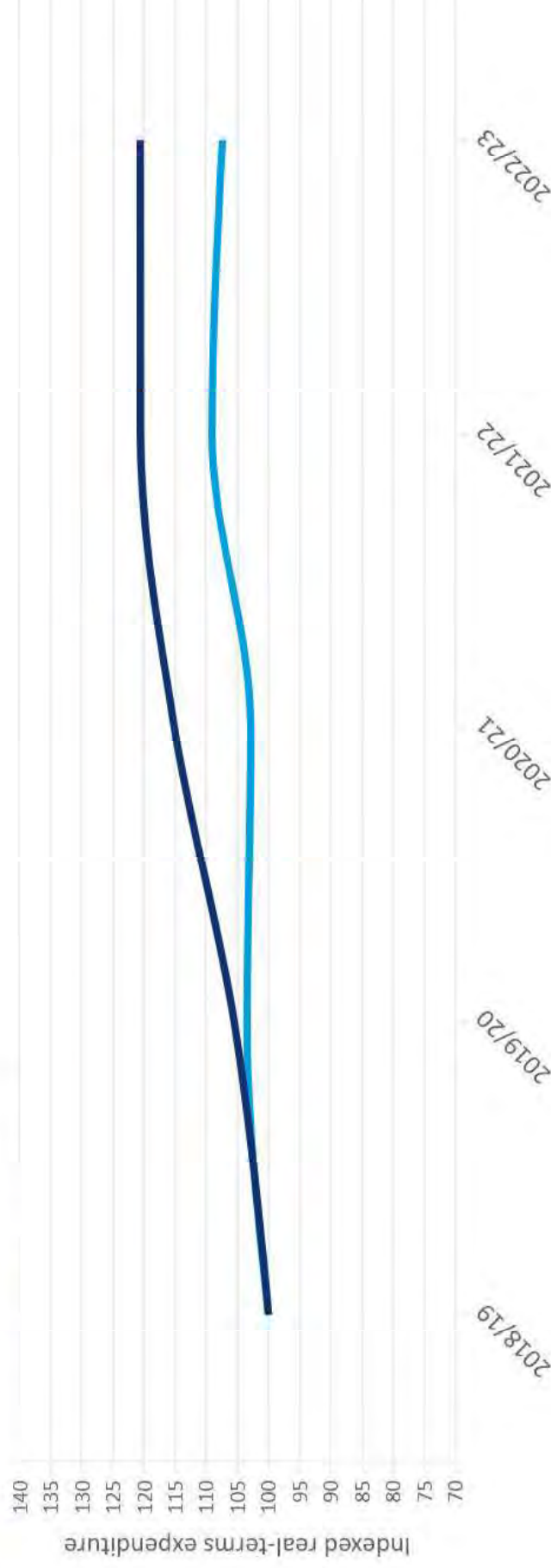
August 2024



Indexed NHS expenditure on mental health in England in comparison to total NHS England expenditure

2018/19 - 2022/23

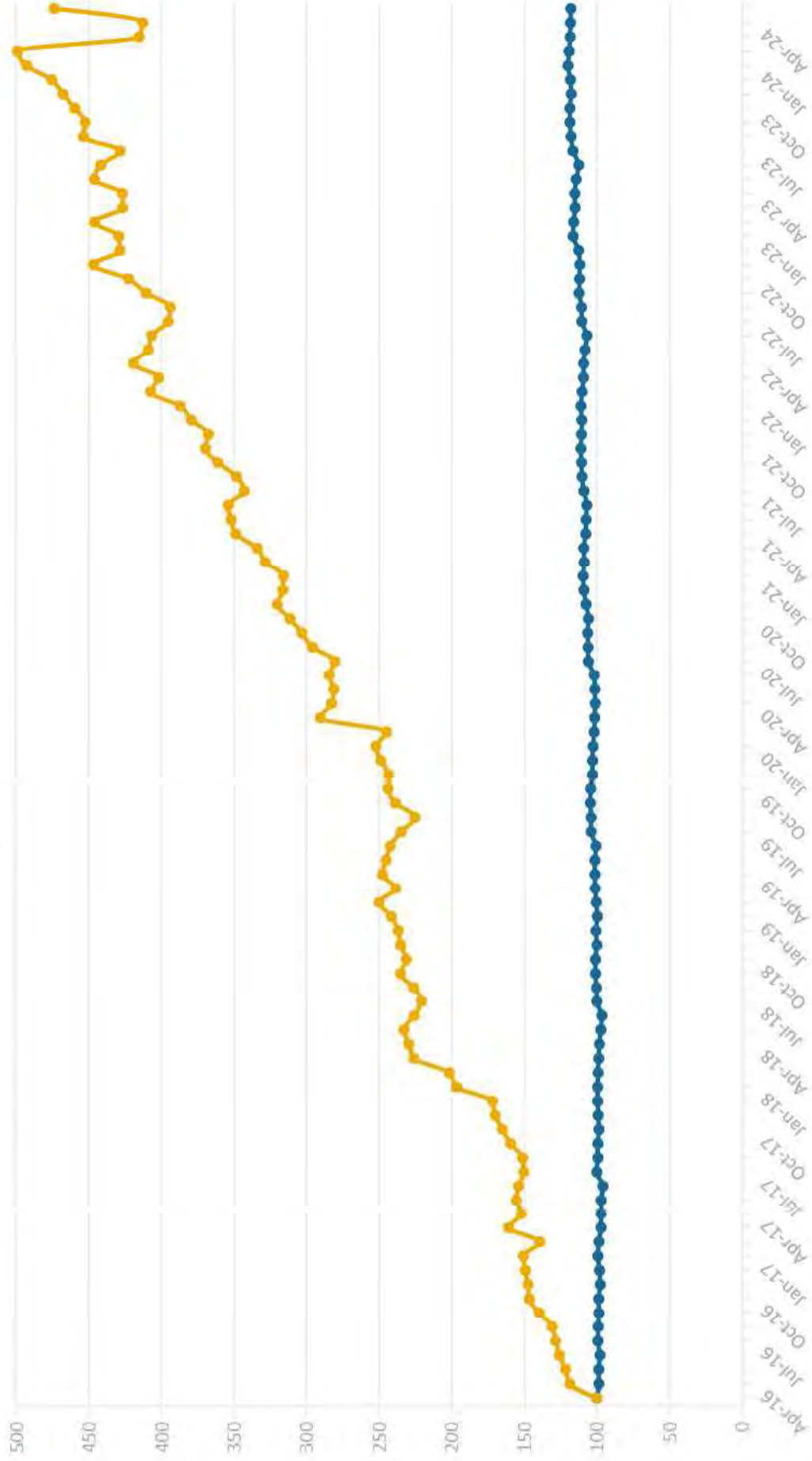
Indexed NHS expenditure on mental health (real-terms)
Indexed total NHS expenditure (real-terms)



Source: BMA analysis of NAO analysis of mental health expenditure, NHS England total expenditure • Real-terms analysis (2023/24 prices) based on GDP deflators at market prices June 2023 (Quarterly National Accounts)
NHS funding is only available on a consistent basis back to 2018/19, hence the data is indexed to then. Mental health funding for 2022/23 is planned, not actual.



Indexed number of FTE doctors in child & adolescent psychiatry
Indexed number of people in contact with children & young people's mental health services



Fitnotes.....

- New DWP legislation July 1 2022
- 5 HCPs now eligible
- Doctors, Physiotherapists, Nurses, Occupational Therapists, Pharmacists
- Majority still done by GP

Fitnotes.....

- Diagnosis / Reason for absence
- No longer need to see patient face to face
- Can be backdated (within reason and if sufficient evidence)
- “May be fit for work” section

Underwriting considerations...

Things to think about....

- Is stress a mental illness?
- Is Neurodiversity an illness?
- Loading vs Exclusions?
- Can a mental health exclusion be used?

Claims considerations...

Things to think about....

- Is there a confirmed mental health diagnosis?
- What assessment has been carried out?
- What treatment / follow up has been arranged?
- Who has issued the fitnote?
- OH assessment – reasonable adjustments?
- Employer vs IME?

Questions and discussion.....



References

- [Key general practice statistics and insights \(rcgp.org.uk\)](https://rcgp.org.uk)
- [Pressures in general practice data analysis \(bma.org.uk\)](https://bma.org.uk)
- [Lothian+Primary+Care+WORKLOAD+toolkit+v12+FINAL.pdf \(squarespace.com\)](https://squarespace.com)
- [Fit note: guidance for healthcare professionals - GOV.UK \(www.gov.uk\)](https://www.gov.uk)
- [Mental health - Care Quality Commission \(cqc.org.uk\)](https://cqc.org.uk)
- [Change NHS: help build a health service fit for the future - GOV.UK \(www.gov.uk\)](https://www.gov.uk)
- [Going Private 2024 - Headline Findings - Independent Healthcare Provider Network \(ihpn.org.uk\)](https://ihpn.org.uk)
- [Our policy work with GPs – Mind](#)
- [Neurodivergent Test | Free Am I Neurodivergent Quiz \(exceptionalindividuals.com\)](https://exceptionalindividuals.com)
- [A transdiagnostic approach to neurodiversity in a representative population sample: The N+ 4 model \(wiley.com\)](https://wiley.com)
- [Autism in England: assessing underdiagnosis in a population-based cohort study of prospectively collected primary care data \(thelancet.com\)](https://thelancet.com)



HelloSelf.

Neurodiversity, Mental Health
and Workplace Challenges

A Psychological Perspective

Dr. Josh Wellman
Consultant Clinical Psychologist

www.helloself.com

November, 2024

Who are HelloSelf?



Who are HelloSelf - At a Glance

- We are a clinically led, technology-driven private psychology provider based in the UK.
- We provide evidence based therapy using our own secure video platform & app.
- We use technology to enhance the therapy experience.
- Over 13,000 members treated to date.
- We have over 300 experienced Psychologists & CBT Therapists.



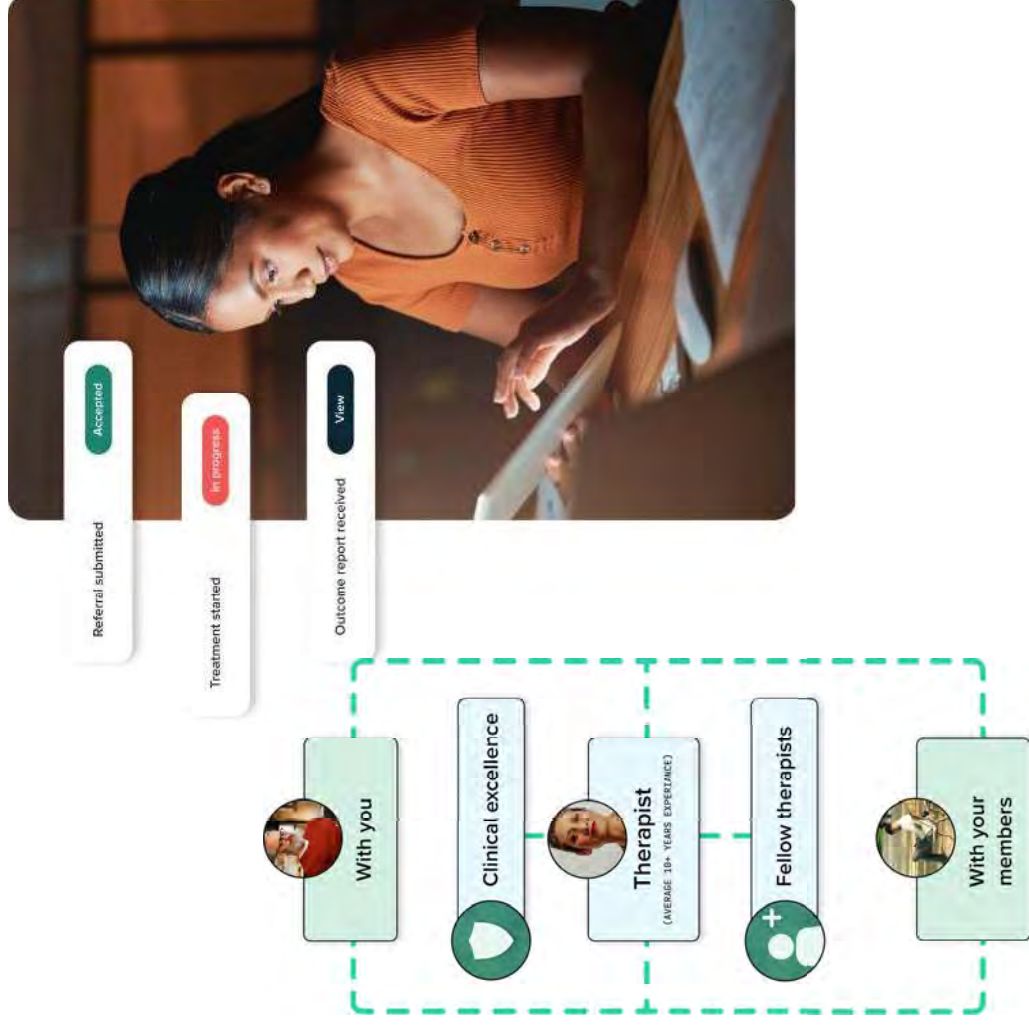
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Working in Partnership with:

- Private medical insurers
- Income protection insurers
- Case Managers
- NHS

Partner Platform

Our partners refer and have ready access to reports, clinical outcomes and secure communication channels with the therapist via our platform.



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What does a typical referral look like?

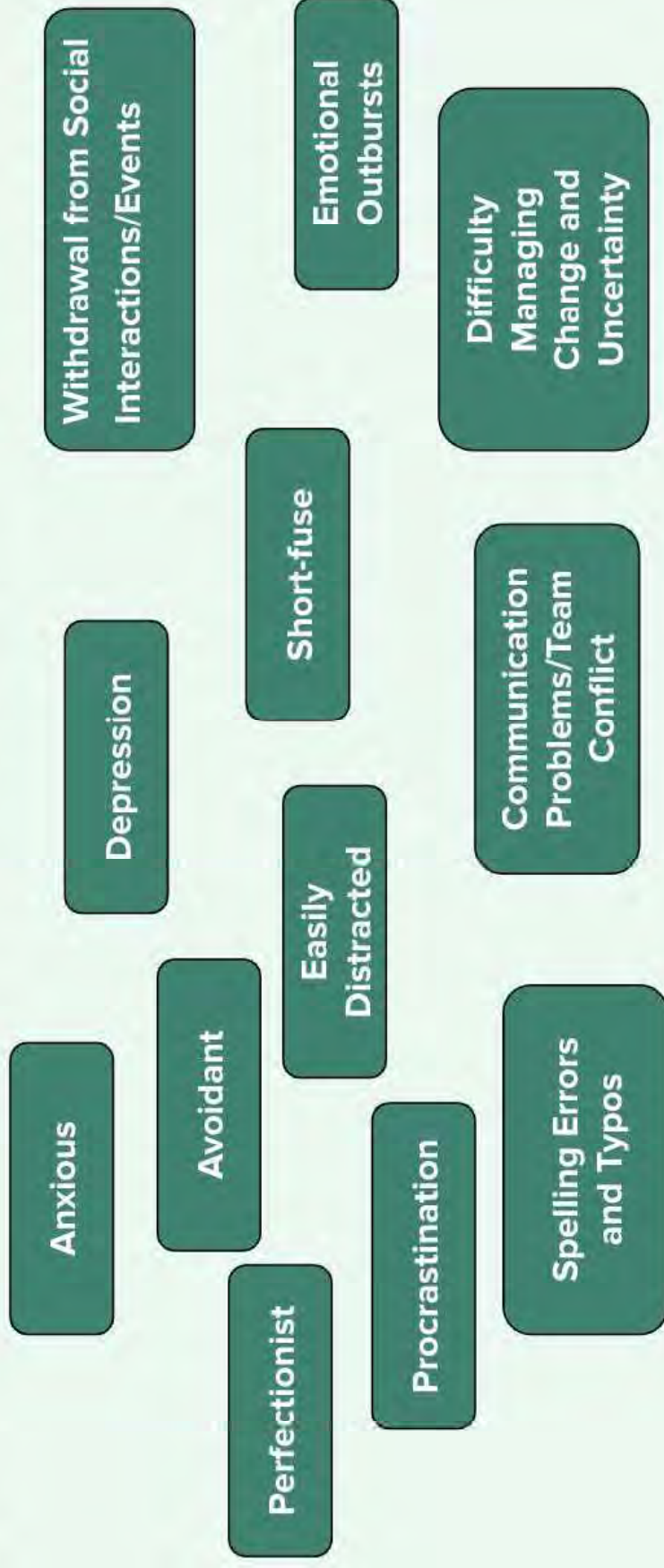
How does Neurodiversity typically present?

1) How could Neurodiversity present?



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2) How could Neurodiversity also present?



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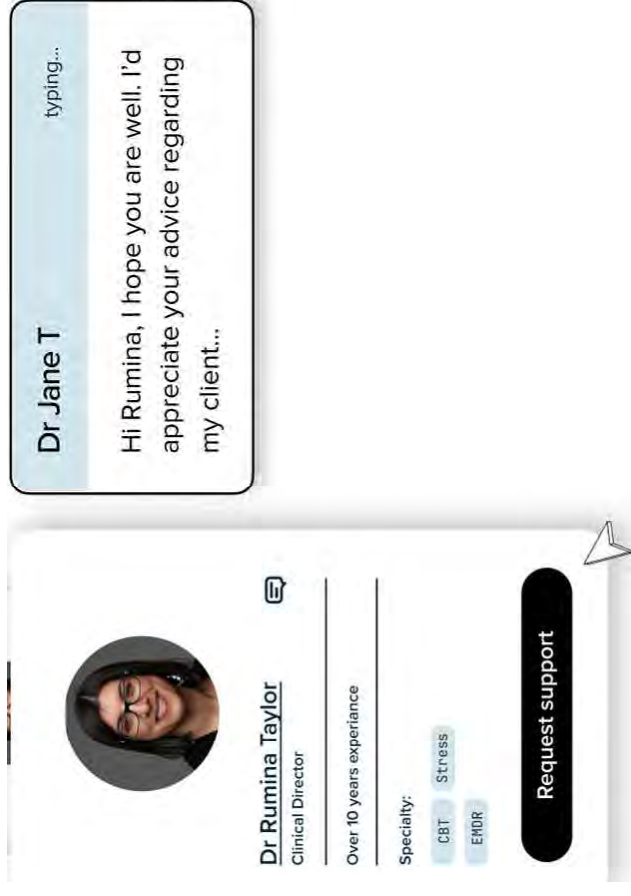
Mental Health Comorbidities

- Rates of mental health problems are significantly higher amongst people with neurodiversity and MH symptoms can be heightened.
- Many patients present initially with mental health concerns - e.g. stress, anxiety, depression.
- Some patients can present with a known neurodiversity diagnosis (e.g. ADHD, Autism), for others this can become apparent through therapeutic conversations.
- Therapy can be an opportunity to open up a conversation around life challenges and links to a suspected neurodiversity diagnosis.
- Therapists can support people with neurodiversity, with or without a diagnosis.

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Typical Income Protection Referral

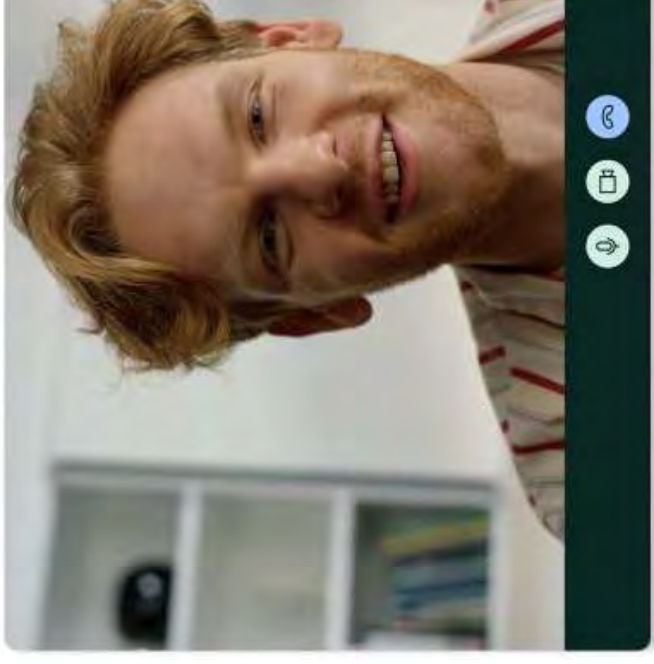
- Co-occurrence of multiple life challenges.
- Signed off work for an extended period.
- High levels of stress, anxiety or depression + neurodiversity.
- Difficulty returning to work.
- Avoidance of going to the office, attending team meetings, return to work meetings.
- Loss of confidence, low self-esteem.
- Feeling overwhelmed at the prospect of returning to work.



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Neurodiversity - Challenges that can Impact Talking Therapy

- Sustaining attention and concentration
- Understanding abstract concepts
- Retaining new information and applying it to everyday life
- Sustaining intensity of eye contact or lengthy conversation
- Difficulty leaving home



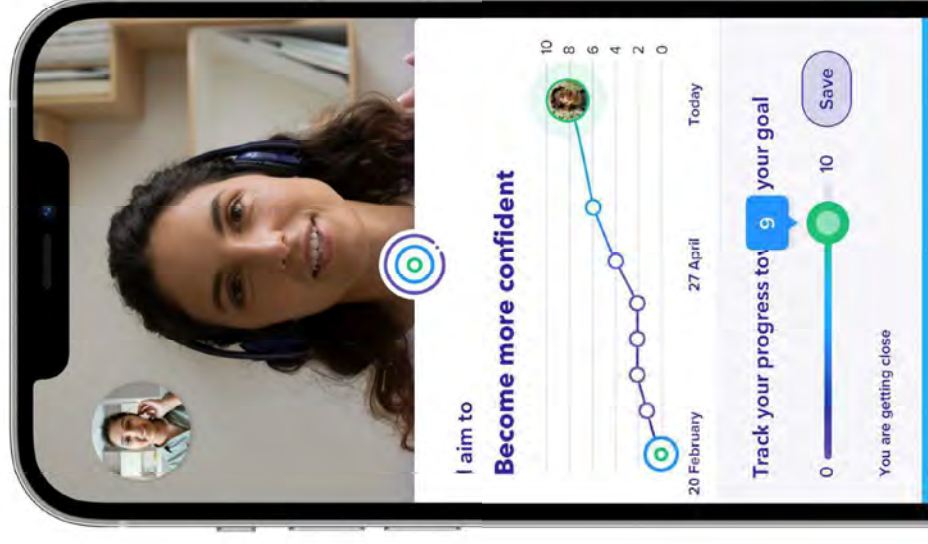
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How we deliver Excellent Outcomes

Inbuilt tech to support neurodiverse members

Tech innovations to support service accessibility and therapeutic success:

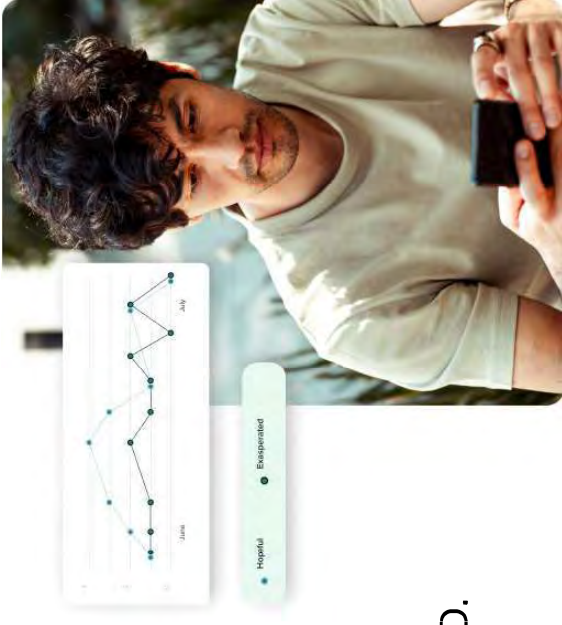
- Clinical matching
- Seen rapidly
- AI session notes and therapeutic recommendations
- Session recordings
- Setting clear goals with achievable outcomes, track progress



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Case Study - Nish

- Signed off with stress following a family bereavement and increased responsibilities at work.
- Previously a high performing employee.
- Experienced symptoms of depression, anxiety - recently diagnosed ADHD.
- Two previous unsuccessful attempts to return to work.
- Matched with specialist ADHD Psychologist.
- Attended 10 sessions with the goal of returning to work and feeling more confident in himself.
- Gained more insight into previous challenges of returning to work.
- Broke down return to work into smaller chunks: phased return, improved communication with manager and HR, learned anxiety coping strategies, planning ahead and doing little and often.
- Worked remotely before returning to the office.



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Prevention and Rehabilitation

- Many people with Neurodiversity thrive in the workplace.
- Preventative interventions such as webinars and coaching can prevent sickness absence and limit claims.
- Psychological therapy interventions can help reduce duration of sickness absence and support rapid return to work.



HelloSelf.

Thank You

HelloSelf.

Therapy and coaching to help you feel and live better.

Neurodiversity in the Workplace

Trends, challenges, and strategies

Dr Stoffel Grobler, Consultant Psychiatrist and
Medical Officer, Swiss Re

Introduction



Concept of Neurodiversity

- Embraces cognitive differences (autism, ADHD, dyslexia, dyspraxia)
- Reshapes approaches to workplace inclusion

Relevance to Key Industries

- Particularly significant in insurance
- Impacts underwriting, policy structures, and disability management

Skills Contributed by Neurodivergent Individuals

- Innovative problem-solving
- Creativity and resilience
- Enhances organizational performance

Organisational Paradigm Shift

- Necessitates inclusive policies
- Supports both neurotypical and neurodivergent employees

Growth in Diagnoses and Contributing Factors

Rising Prevalence of Neurodivergent Diagnoses

- Significant increase in neurodivergent diagnoses in both children and adults
- **ADHD Diagnoses:** Nearly 20% rise in adult ADHD diagnoses over the last decade (ADHD Foundation)
- **Autism Diagnoses:** Increasing at a rate of 7-10% annually

Impact on Workforce

- **2024 Birkbeck Report:** Up to 15-20% of the working population may be neurodivergent
- Highlights widespread presence of ADHD, autism, and other conditions in workplaces



Contributing Factors to Increased Diagnoses



Better Awareness and Understanding

Increased public awareness and destigmatization encourage individuals to seek diagnoses
Advocacy and education empower self-recognition of neurodivergent traits



Expanded Assessment Boundaries

Broader diagnostic criteria identify a wider range of traits
Enables recognition of subtler symptoms that may have been undiagnosed previously



Evolving Diagnostic Criteria

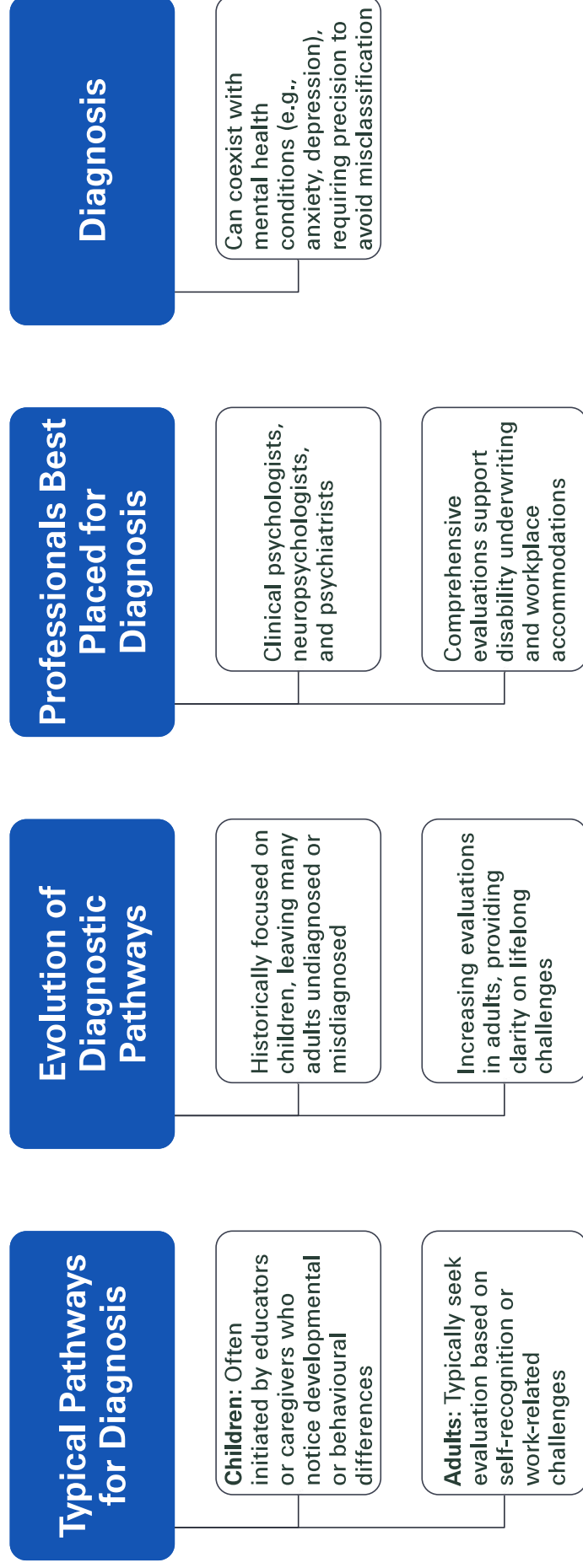
DSM-5 and ICD-11 updates reflect inclusive understanding
Recognize diverse manifestations of conditions like ADHD and ASD across ages



Increased Screening in Schools and Workplaces

Proactive screening allows for early identification and timely support
Enables individuals to understand and seek accommodations for neurodivergent traits

The Path to Diagnosis



Concerns Around Potential Over-Diagnosis

- **Rising Diagnoses and Over-Diagnosis**
 - Expanded diagnostic criteria and social pressures may lead to misclassification
 - Unnecessary labelling can complicate policy definitions and increase healthcare costs
- **Pressures on Clinicians**
 - Growing demand for inclusive accommodations can pressure clinicians to diagnose
 - Especially prevalent in educational and workplace settings
- **Implications for Insurers**
 - Need for evidence-based approaches to avoid overextended criteria
 - Balancing accurate support while preventing unnecessary cost escalations



Distinct but Interconnected

Neurodiversity vs. Mental Health

- Neurodiversity is separate from mental health conditions
- However, co-occurring mental health challenges are common (e.g., anxiety, depression)

Workplace Challenges

- Misalignment with neurodivergent needs or lack of support can increase mental health difficulties
- Highlights the importance of neurodiversity-informed Employee Assistance Programs (EAPs)

Implications for Insurers

- Understanding the neurodiversity-mental health intersection is crucial for policy design
- Comprehensive policies should address both cognitive and emotional well-being
- Important for effective disability management and accurate claims evaluation

Neurodiversity in ICD and DSM Coding – Official Recognition

- **ADHD and ASD**

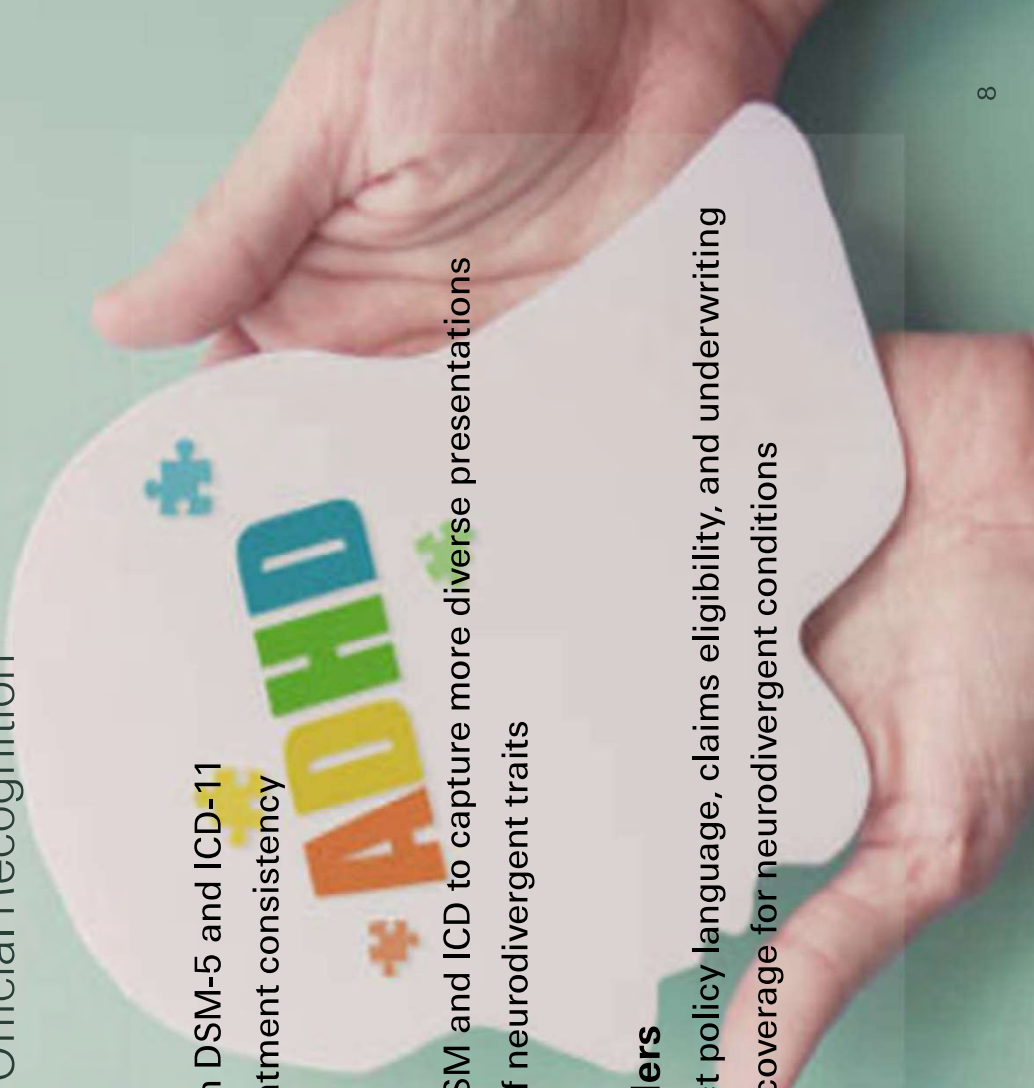
- Both conditions are officially recognized within DSM-5 and ICD-11
- Standardised coding aids in diagnosis and treatment consistency

- **Evolving Classification Systems**

- Growing awareness may prompt updates to DSM and ICD to capture more diverse presentations
- Updates expected to better reflect the range of neurodivergent traits

- **Implications for Insurers and Healthcare Providers**

- Need to monitor coding updates as they impact policy language, claims eligibility, and underwriting
- Accurate coding adjustments ensure relevant coverage for neurodivergent conditions



Co-Morbid Conditions and Functional Impacts

- **Anxiety and Depression**
 - Frequently co-occur with neurodivergent conditions
 - Complicate the impact of neurodivergent traits and affect coping capacity
- **Functional Impact in Workplace Settings**
 - High demands may exceed neurodivergent individuals' coping abilities, exacerbating co-morbid symptoms
- **Insurance Implications**
 - Recognizing the neurodiversity-co-morbidity interplay aids in more effective claims management
 - Supports comprehensive, holistic accommodations in workplace policies



Impact of Receiving a Diagnosis in Adulthood

- **Adult Diagnosis Benefits**
 - Provides validation and empowers individuals to seek accommodations
 - Encourages self-advocacy in navigating workplace needs
- **Challenges in Workplace Accommodations**
 - Workplaces may lack readiness to support neurodivergent needs for adult-diagnosed employees
 - Securing accommodations can be challenging in unprepared environments
- **Insurance Considerations**
 - Adult diagnoses impact disability underwriting as they increase accommodation demands
 - Insurers should be aware of this trend to meet future support needs



- **Unique Neurodivergent Needs**

- Sensory overload, rigid schedules, and limited support present difficulties
- Typical environments may not align well with neurodivergent employee needs

- **Adjustments for Engagement and Productivity**

- Noise-cancelling tools, quiet spaces, and flexible work arrangements can improve experience
- Inclusive culture reduces turnover and lowers disability claims

Barriers and Opportunities

- **Limited Accommodations and Misunderstandings**
 - Lack of accommodations and colleague misunderstandings hinder neurodivergent contributions
 - Inflexible structures limit potential for effective engagement
- **Opportunities for Strengths**
 - Neurodivergent employees bring strengths in creativity, problem-solving, and attention to detail
 - With supportive environments, these strengths can drive workplace innovation
- **Insurance Perspective**
 - Recognizing barriers and opportunities aids accurate disability claim assessments
 - Positive employment outcomes are supported by targeted accommodations



Key Findings from the 2024 Birkbeck University Neurodiversity Report

- **1. Well-being Challenges**
 - Neurodivergent employees report lower well-being than neurotypical colleagues
 - Common issues: mental health challenges, sleep disturbances, and sensory overload
- **2. Diverging Workplace Perceptions**
 - Neurotypicals report higher psychological safety and job satisfaction
 - Neurodivergent employees often feel pressured to “mask” their challenges
- **3. Psychological Safety and Performance**
 - Decline in psychological safety for neurodivergent employees since 2023
 - Perceived unfairness and limited adjustments impact career satisfaction
- **4. Gaps in Broader Organizational Support**
 - Direct manager support rated positively, but broader support (conflict resolution, inclusivity training) is lacking
 - Rising perceptions of unfairness highlight the need for comprehensive inclusion policies.



Recommendations for Neuroinclusion

Goal: Foster an inclusive workplace that supports engagement, productivity, and retention for neurodivergent employees.

1. Job Redesign

- Develop roles that leverage neurodivergent employees' strengths without rigid job descriptions.

2. Specialized Career Paths

- Offer career paths that emphasize neurodivergent skills, fostering meaningful contributions.

3. Preventive Well-being Programs

- Implement quiet zones and flexible work arrangements to alleviate cognitive and sensory demands.



Key Insights and Benefits from Deloitte

**Focus on systemic changes to support neurodivergent employees.
Emphasis on adaptable work cultures where neurodivergent individuals can thrive**

- **Definition and Scope of Neurodiversity**
 - Judy Singer's framework views cognitive differences as strengths
 - Neurodivergent employees contribute through creative problem-solving and analytical skills
- **Growing Awareness and Demand**
 - Millennials, Gen Z, and parents of neurodivergent children are driving neuroinclusion
 - Increasing pressure on organizations to adopt neuroinclusive policies
- **Business Advantages**
 - Enhanced productivity and retention
 - Example: JPMorgan Chase sees high productivity from neurodivergent employees in tech roles
 - Neuroinclusive practices (clear communication, flexible schedules) benefit all employees



Challenges in Establishing Neuroinclusive Workplaces - Barriers to Neuroinclusion

Outdated Mindsets and Stereotyping

- Limited definitions of neurodiversity restrict opportunities
- Some organizations assign roles based on perceived limitations, reinforcing stereotypes

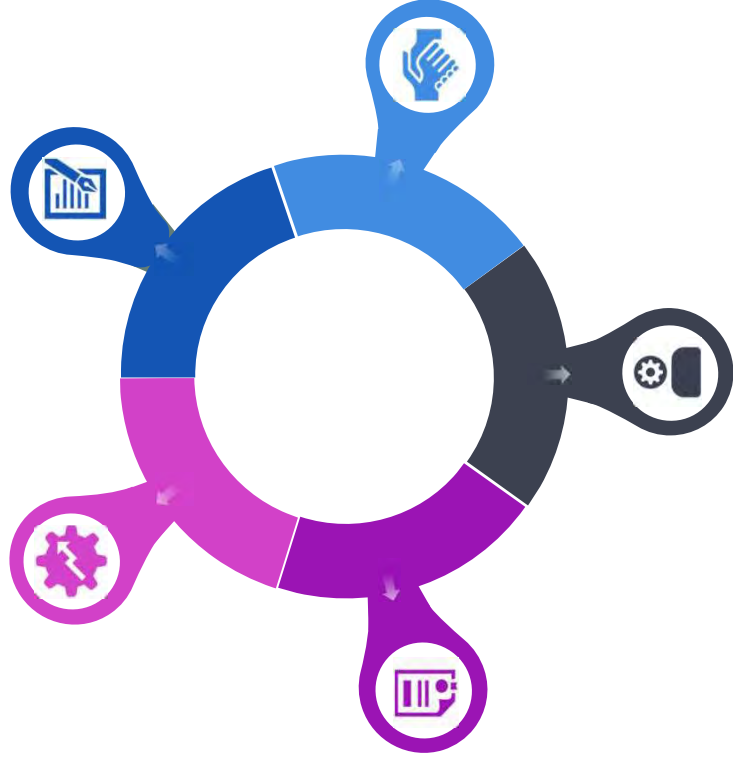
Trust and Self-Disclosure Issues

- Neurodivergent employees may feel the need to “mask” traits to fit expectations
- Establishing trust is crucial to create safe environments for self-expression

Need for Change

Cultivating a neuroinclusive culture requires addressing these barriers and encouraging open communication

Designing a Neuroinclusive Organization – Key steps for creating a neuroinclusive environment



Plan and Design

Develop systems catering to neurodiverse needs (e.g., flexible schedules, structured processes)

Hire and Build Trust

Focus on skills-based hiring, ensuring fair evaluations

Individualise Roles

Design roles that utilise individual strengths, offering career growth opportunities

Coach and Develop

Implement mentoring programs and create resource groups for community support

Measure and Adapt

Use metrics to assess effectiveness and refine neurodiversity programs based on feedback

Goal: Support engagement, reduce disability claims, and benefit insurers and organizations alike

Implications for Insurers and Reinsurers

Policy Design and Underwriting - Adapting Policies

- Address rising neurodivergent diagnoses with inclusive policies
- Define disability criteria that encompass cognitive and emotional challenges
- Aim for comprehensive coverage and minimize disputes

Disability Claims Management - Understanding Support Needs

- Tailor solutions for neurodivergent employees, such as accommodations for sensory sensitivities
- Align with neurodivergent needs to improve claims management and reduce costs

Risk Assessment and EAPs - Evaluating Neuroinclusive Practices

- Gain insights into employee retention and stress through assessment
- Expand Employee Assistance Programs (EAPs) with neurodiversity-informed resources like counseling and assistive technology
- Foster positive outcomes while minimizing claims

Conclusion - Embracing Neurodiversity as an Asset



Cultural and Performance Benefits

Recognizing neurodiversity enhances workplace culture, performance, and retention
Neurodivergent employees bring innovation, resilience, and diverse perspectives



Value for Insurers and Companies

Neuroinclusive policies foster environments where neurodivergent employees can excel
Drives both creativity and inclusivity, benefiting organizational outcomes



Creating Thriving Workplaces

Embedding neuroinclusive practices supports long-term success
Beneficial for both organizations and insurers, promoting thriving, inclusive work cultures

Any questions?





Thank you!



Dr Stoffel Grobler
Consultant Psychiatrist
and Medical Officer, Swiss Re

Follow us



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3. **ADHD Foundation.** (2023). *Annual Report on Neurodiversity Trends*.
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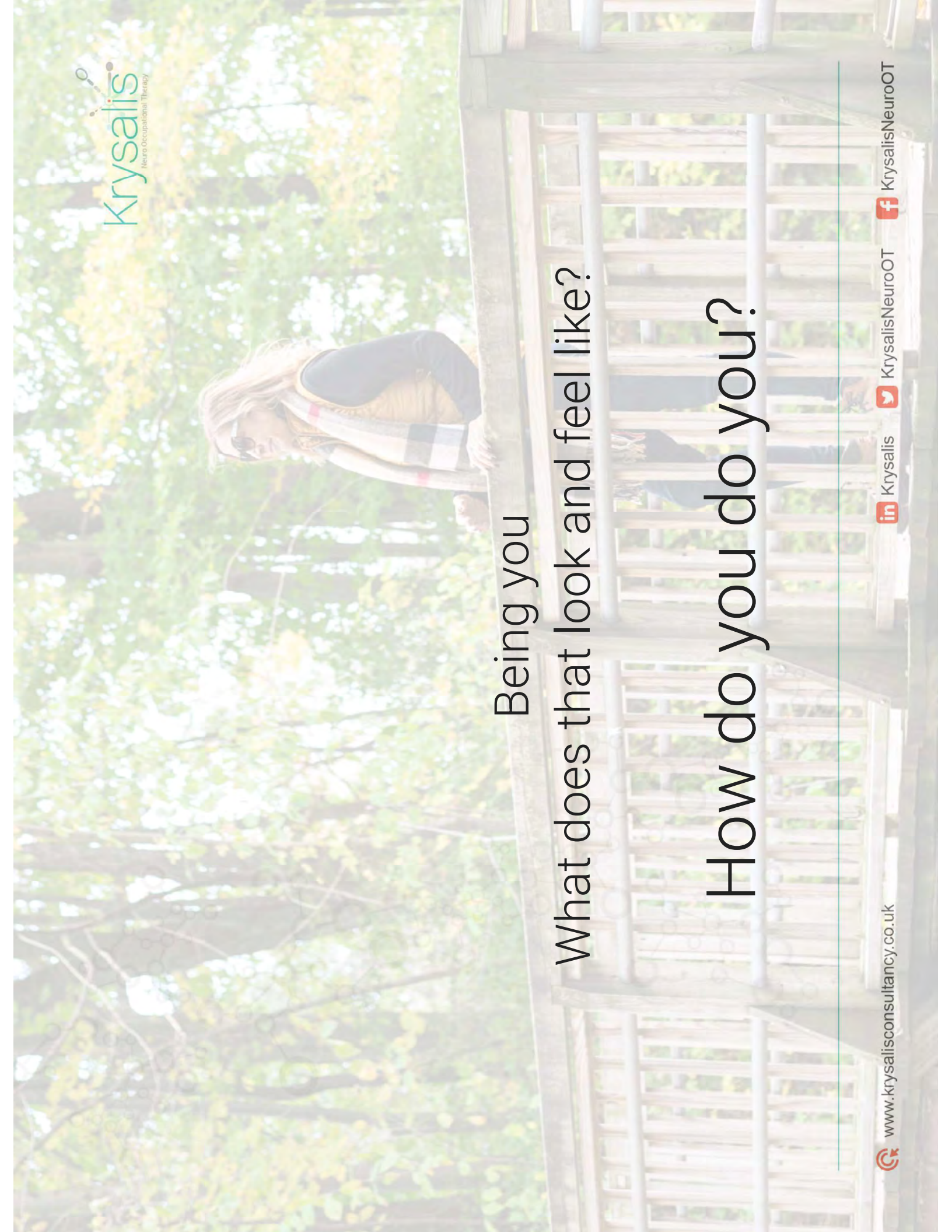
neuro:logical
that makes sense.



‘Now you see me – the power of
curiosity’

Jo Throp CEO and Founder
November 2024

realising potential, transforming lives



Being you What does that look and feel like?

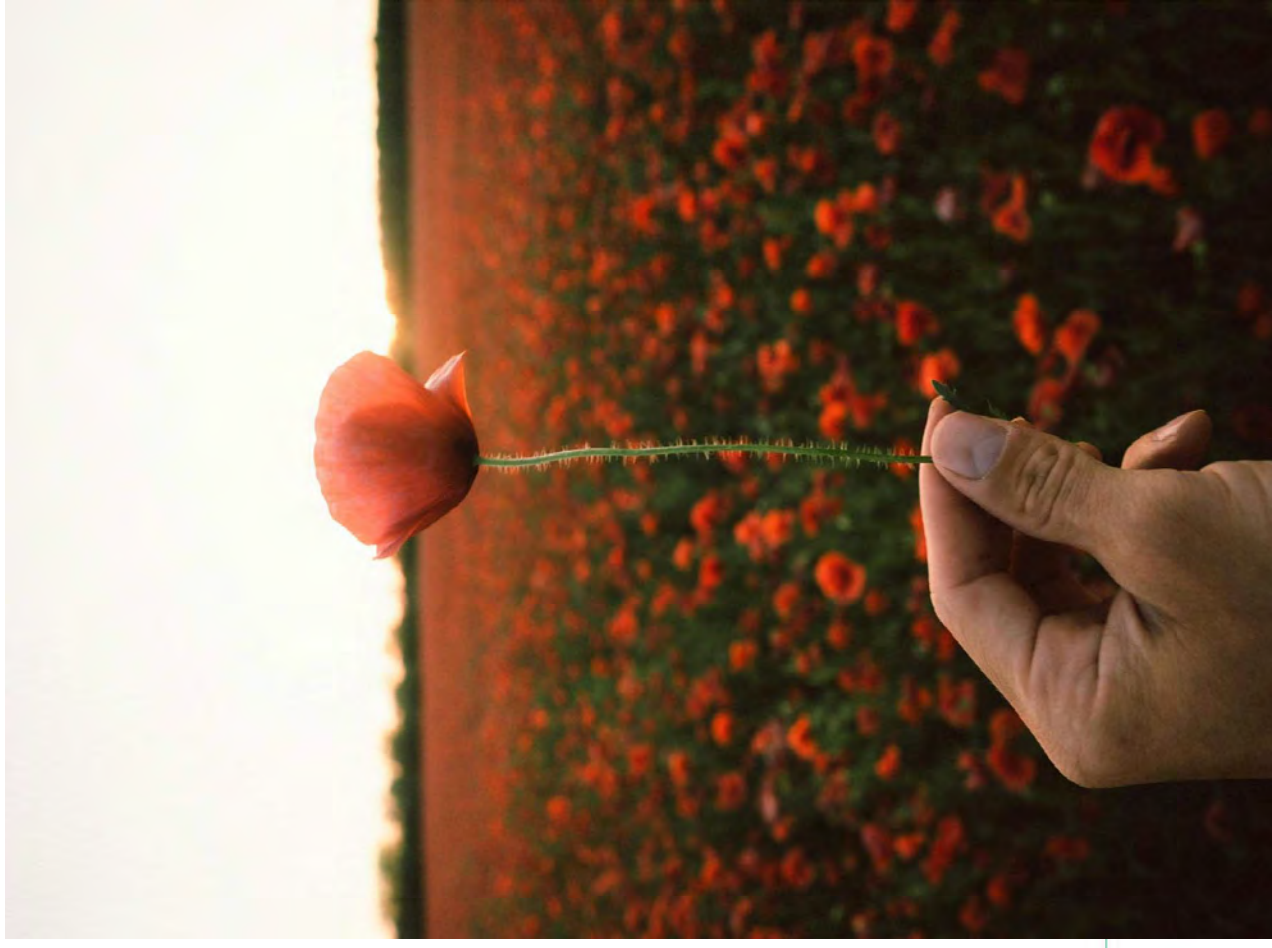
How do you do you?

What do people living with neurodivergence need?

What solutions are the workplace looking for that impact work activities and productivity?

These questions matter because from a neuroscience perspective, this is neuroplasticity in action

- Differences does not mean diagnosis, although it might.
- Differences does not mean you, and not me.
- Differences is our uniqueness and from time to time our needs.



What we do is the practical stuff, neurological vocational rehabilitation



The occupational therapy role is to **assess function and performance** and create a formulation to bring alive the experience of living and doing at work.

Away from mental health to **performance and participation**



What we will cover

- An **occupational therapy perspective** shifting the lens from mental health to function.
- Living experiences to **bring alive the complexity of human function**.
- Neurodivergence, neuroscience and everyday function
- The experience of differences using case examples to bring a **personal perspective**
- Layers of challenges and **how things escalate**
- **Performance barriers** that impact success at **work and doing life with others** in the work environment

Adopting a mindset of curiosity is helpful, enabling us to consider more broadly an individual's needs and their potential.



Curiosity, or the lack of it, speaks right to the heart of our combined potential to cultivate a diverse, valuable and productive workforce.



So how do we foster greater understanding between people to influence things for the better?

We know that **curiosity** as a psychological trait is linked to genetics, however it is the environment that cultivates and benefits from a curious mind



The word embodied refers to the idea that your **thoughts, emotions** and **behaviours** are intrinsically connected to your body and experiences.

The view you have of the world is unique to you, your **experiences** form and shape you.

What I do is Me!

This certainty is gained from a clever little process of brain wiring



Neuroscience suggests that a substantial portion of our brains processing is automatic;

- Up to **95%** of our cognitive activities are automatic
- **40 – 50%** of our daily actions are habitual
- Est. **80 – 95%** of our sensory processing and initial decision making occurs unconsciously
- Many emotional reactions are automatic

...challenged more neurologically and functionally

...functional abilities fluctuate because the neurological demands are greater

...complexity of tasks and environment will impact function, performance and wellbeing

Be grateful for the ease of the ordinary...

These experiences maybe persistent or heightened in their intensity by change, a life event or trauma

Imagine....
...challenges with your thinking skills,
your emotions, your communication
and behaviour at work.

...if understanding and being
understood was not something you
could rely on

The consequence reduced certainty, reduced
confidence in functional capabilities or belief in their
ability to influence the situation

‘How do you do you?’

Case example



Diagnosed as **autistic** as an
adult

Functional neurological
disorder

Fall in workplace;
concussion



occupational therapy approach
neurological vocational rehabilitation
service

Case example



...I would start tasks off well yet would become increasingly unhappy. I had a clumpy walk and childlike emotions, and I grew afraid of where this was leading.

I couldn't understand the reactions I received from the very people who should have been 'keeping an eye' and ensuring all's well. They seem to be critical, and rejecting, as if I were a threat.

I felt increasingly inadequate, less of a human being. I didn't seem to belong in the world, they were feeling intimidated by the abilities I didn't know I had, and a bluntness which I thought was just honesty.

I kept trying to be helpful, and to please, and all this did was aggravate the situation. So, then I'd develop a chip on my shoulder then resent the treatment I came to expect.



Diagnosis of Autism, concussion and FND

To try and understand her personal perspective and therefore, her experiences

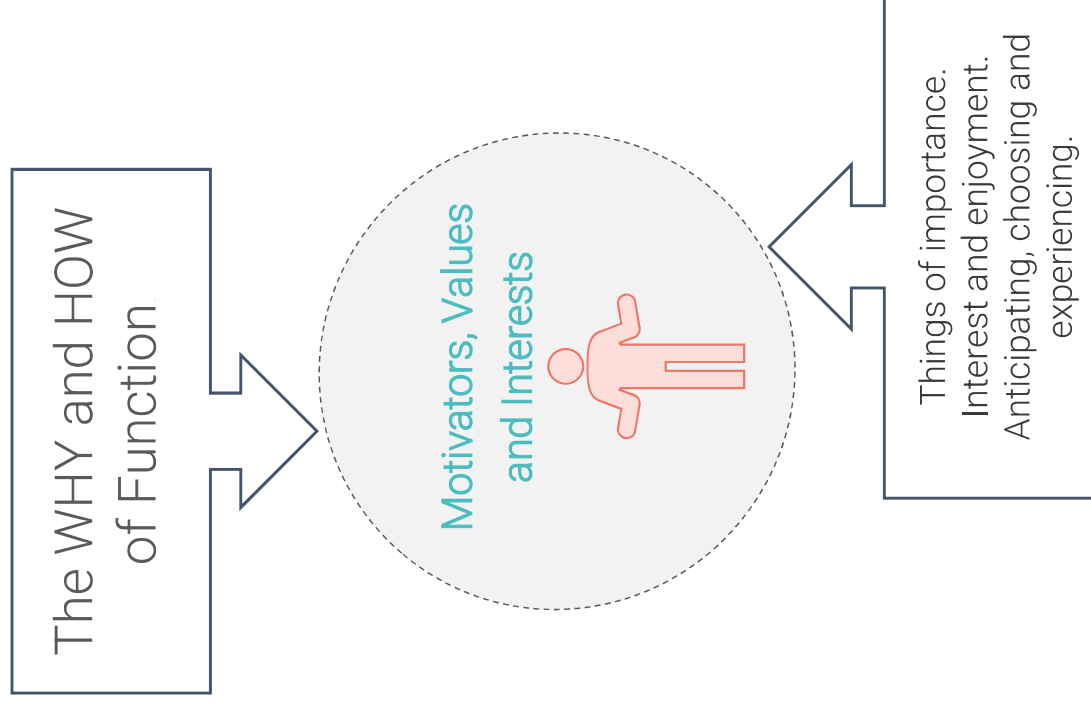


I kept trying to be helpful, and to please.

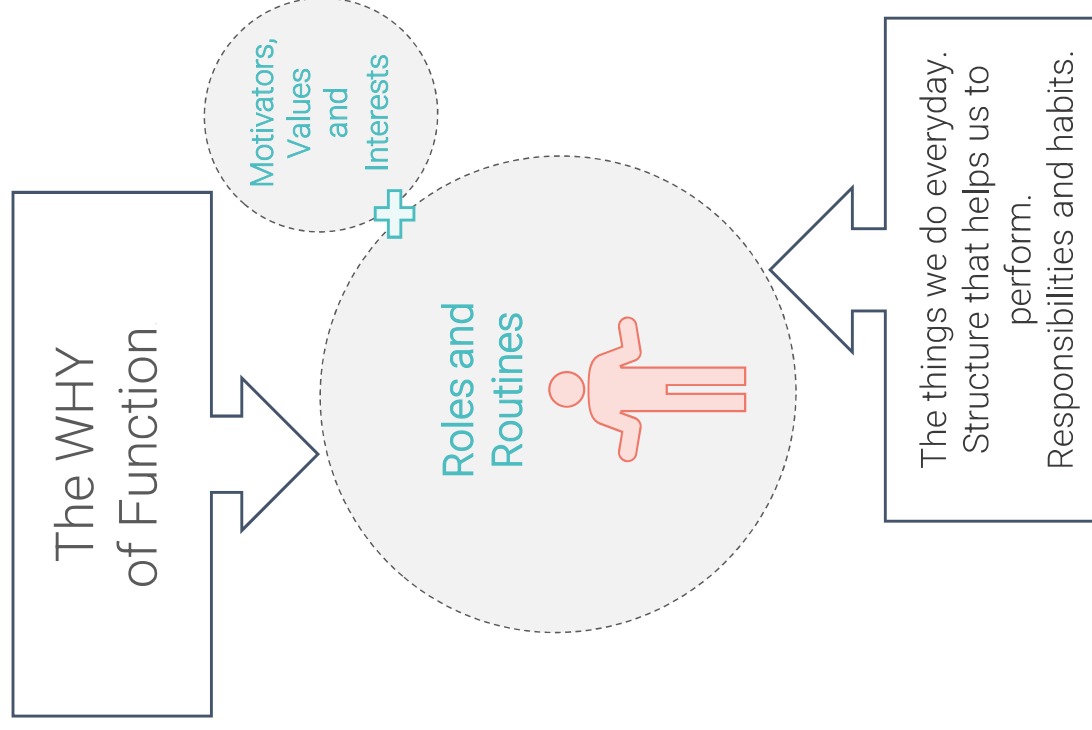
I felt increasingly inadequate, less of a human being.



Innate desire to serve is a key motivator.
Inability = reduced motivation



Exploring experience of routines and roles

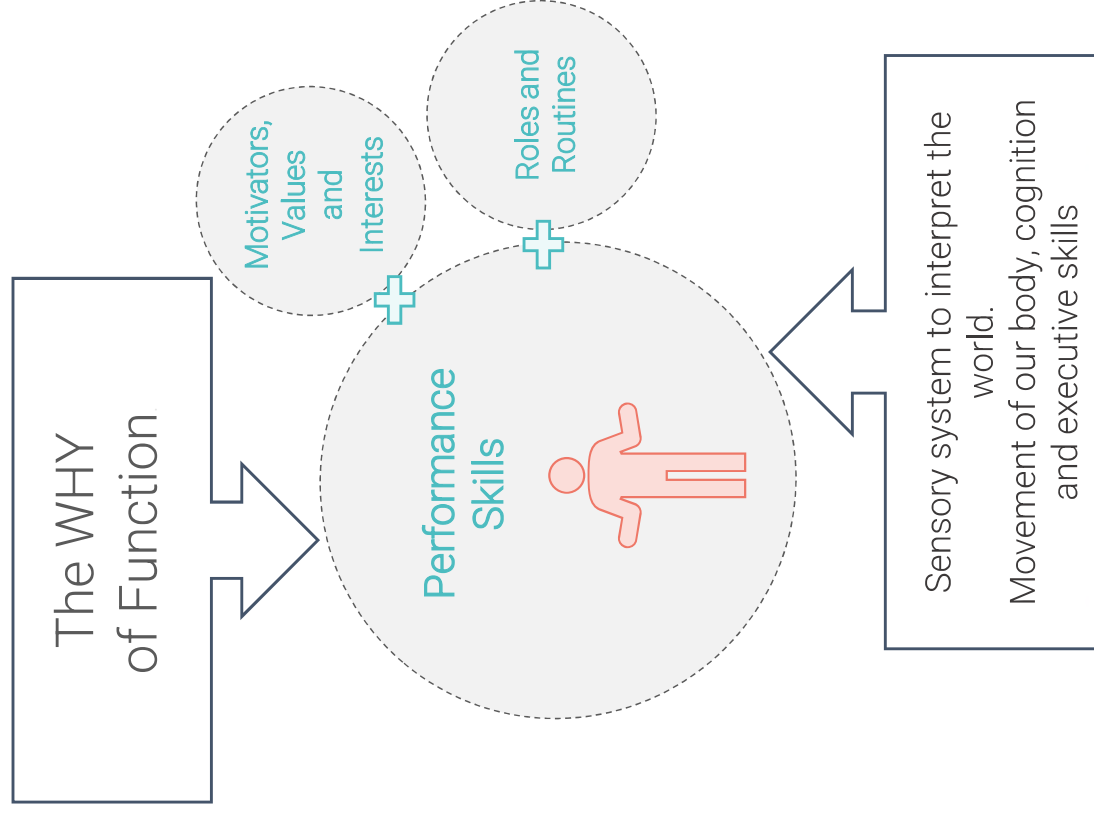


*I would start tasks off well,
yet would become
increasingly unhappy.*



Experiences of
performance.
Starting with
commitment but
ultimately dissatisfied.

Exploring experience of performance skills



66

*I had a clumpy walk and
childlike emotions.*

22

Belief about how she is
viewed.
Judgement regarding
abilities have an impact
on performance and
motivation

Exploring experience of the environment



They seem to be critical, and rejecting, as if I were a threat.

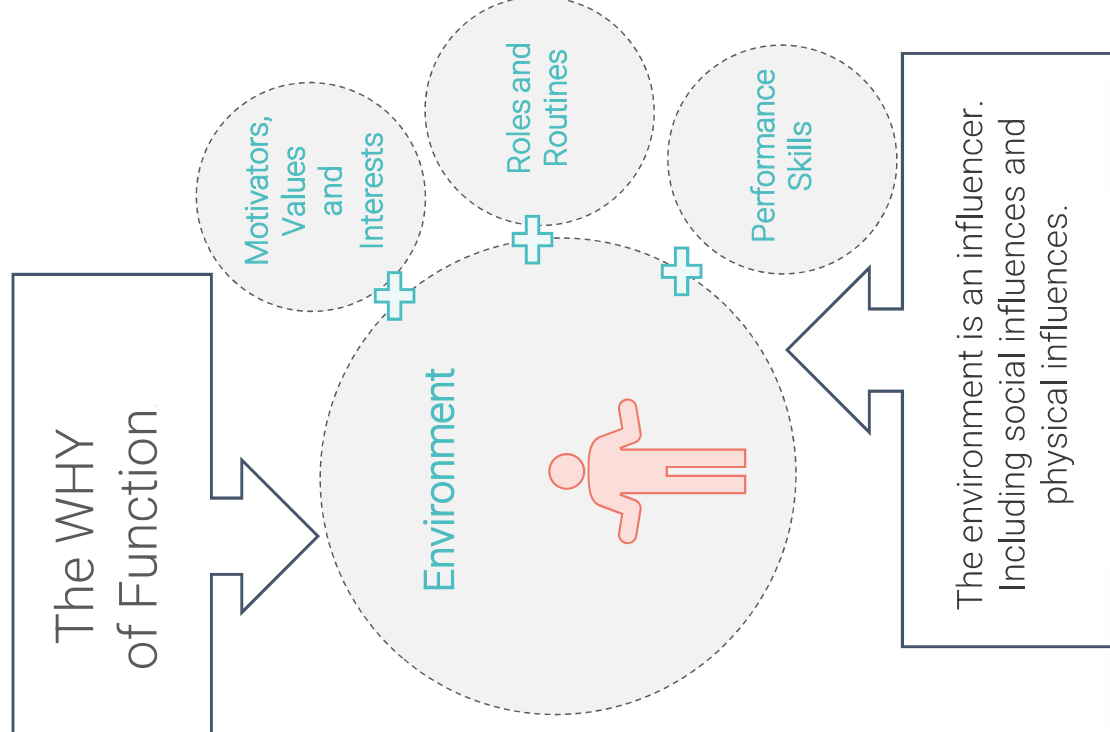
So, then I'd develop a chip on my shoulder then resent the treatment I came to expect.



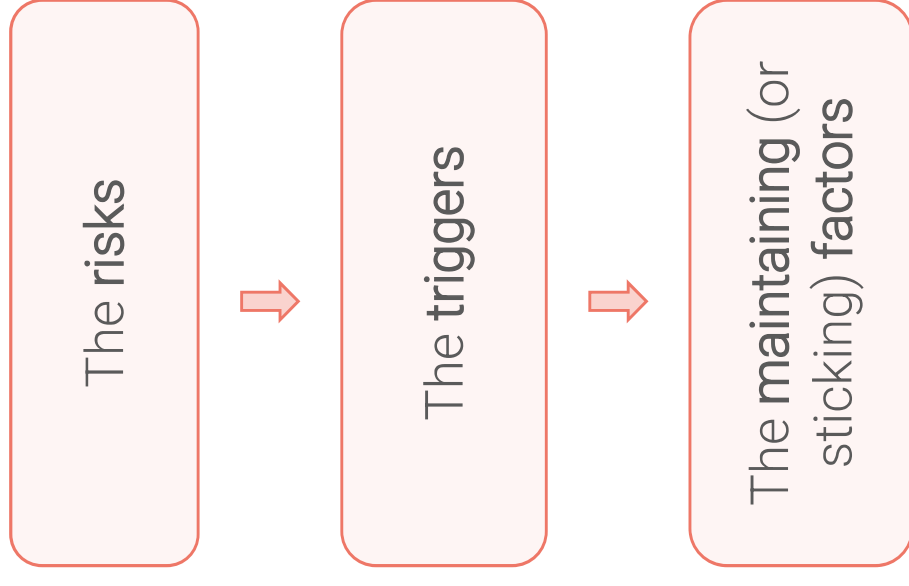
Perception of social relationships at work.



Long term sick leave



Using the biopsychosocial model as a framework for the clinical observations



Using the biopsychosocial model as a framework for the clinical observations



The risks



The triggers

The maintaining (or sticking) factors

Past and recent adverse life events, personality traits, comorbidities;

- Perfectionist and high academic achiever
- Interested and caring nature
- History of feeling socially awkward

Using the biopsychosocial model as a framework for the clinical observations

The risks

The triggers

The maintaining (or sticking) factors



A trigger could be a change or transition or injury or illness;

- Fall at work and concussion a few years prior to our involvement
- Her role required periodic resettlement, planned for 18 months = increased anxiety
- New line manager

Using the biopsychosocial model as a framework for the clinical observations

The risks

The triggers

The **maintaining** (or
sticking) factors



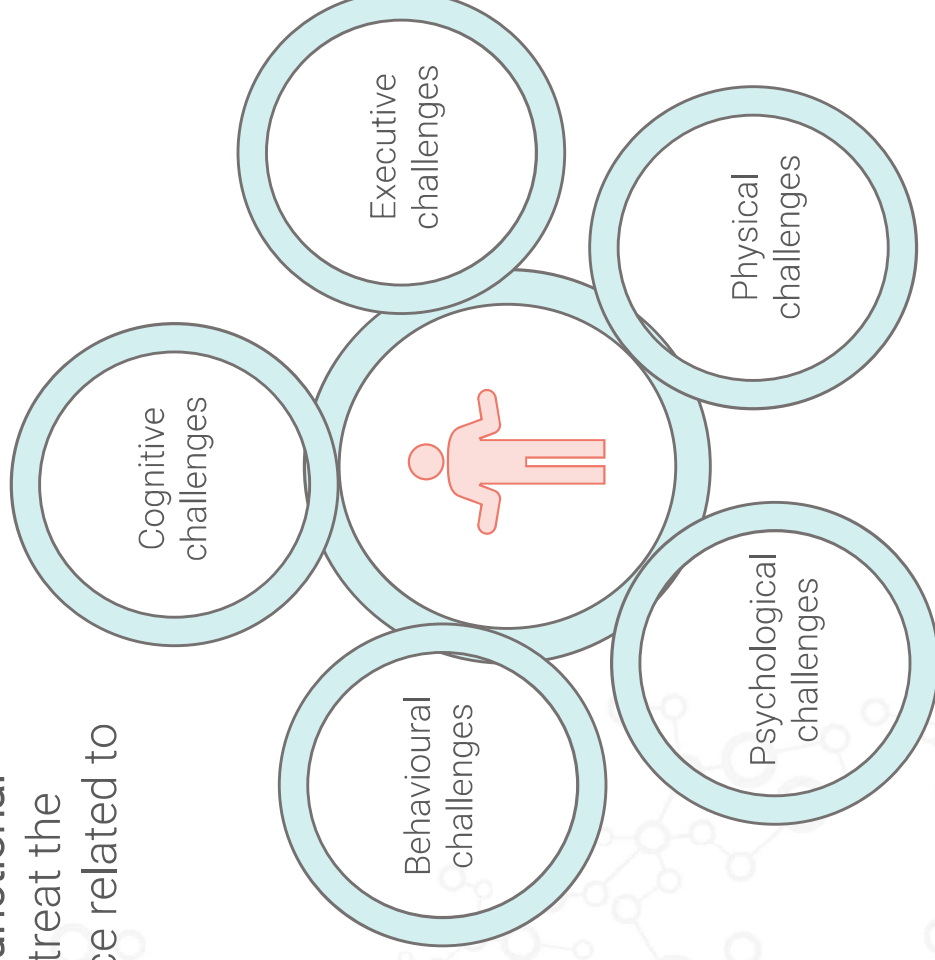
The factors that keep a person stuck;

- Lack of practical response meaning challenges festered
- Functionally deconditioned
- Gradual reduction in roles and responsibilities over a period of time
- Withdrawing socially outside of work
- Mistrust of technology
- Neuro symptomology impacting performance with physical, cognitive and executive function

Our primary role was to address her symptoms and then work towards building trust and understanding



As an occupational therapist I can make **a judgement on functional ability**. We assess and treat the barriers that people face related to their diagnosis.



Social communication challenges

Physical communication needs

Behaviour, insight and awareness

Physical limitations & tone problems

Co-ordination difficulties

Altered physical sensation

Medically unexplained symptoms

Hidden disability

Memory

Attention

Fatigue & sleep disturbance

Visual perception & processing challenges

Auditory & vestibular functions

Executive skills

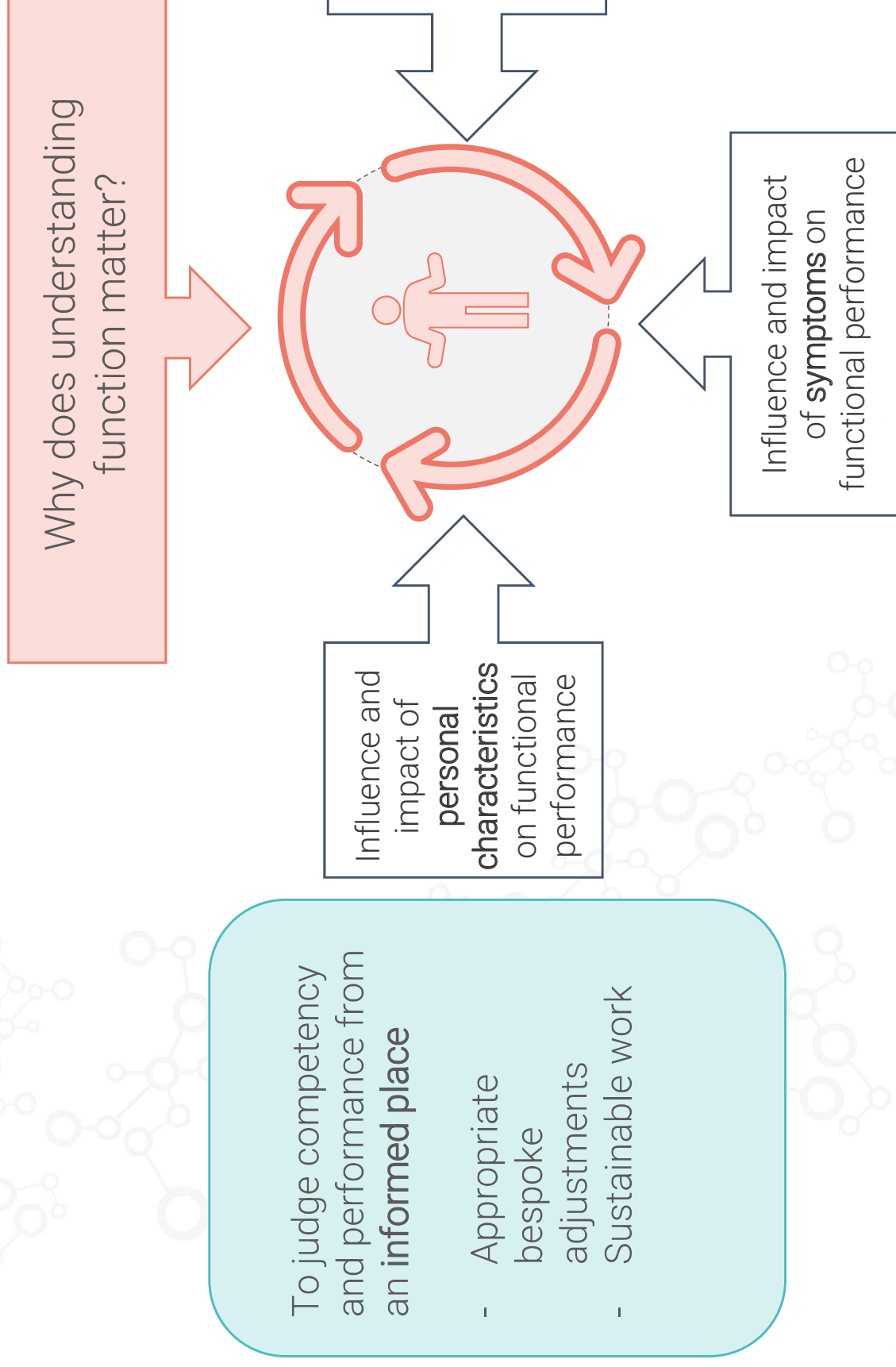
Sensory needs

We consider all these things in the context of function and performance related to work activities, tasks and important roles

We provided a clinical formulation to **bring alive the experience of** living and doing at work.

Helping all parties to understand the **functional implications**, or the things that have been observed in the workplace, and what can be done to **minimise or remove challenges and capitalise on strengths**.

Looking through the lens of a neurological occupational therapist at Krysalis



In this quote she described the experiences of being asked to take on a new project when unwell

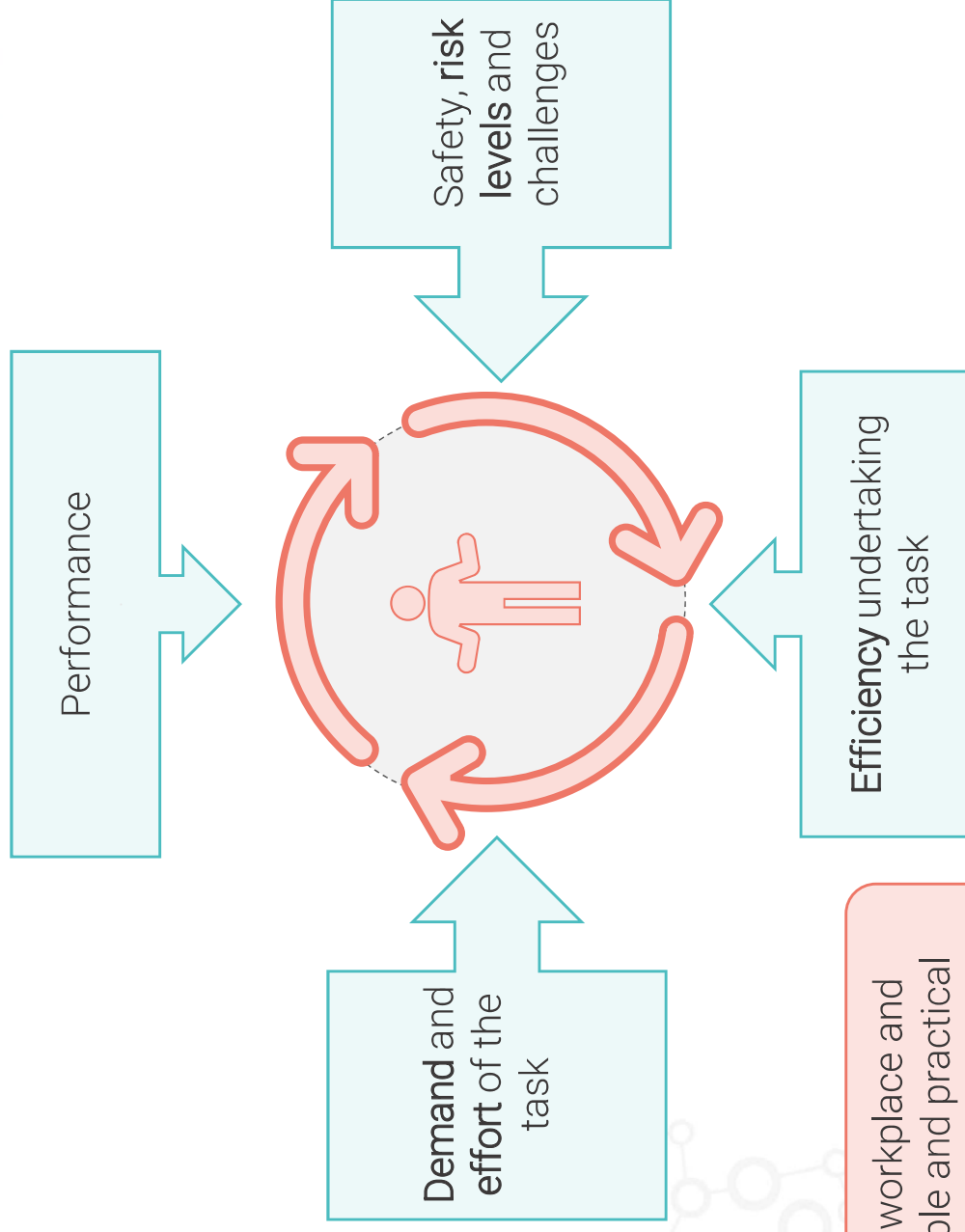


...It (the project) was like a van dumping a lorry load of Lego on your front lawn and someone saying well there is a space station and a pirate ship in there.

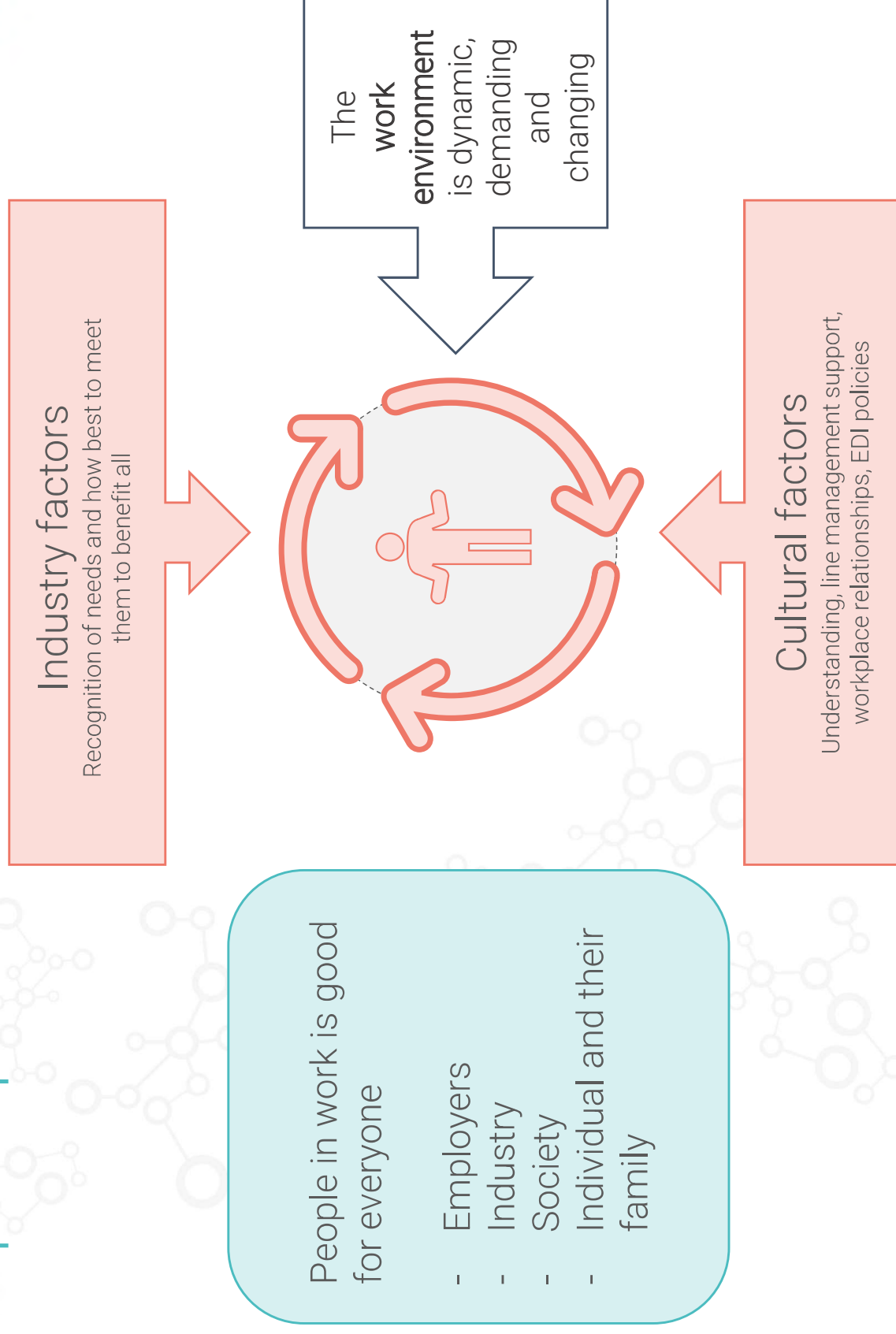
So, you ask where the instructions are, and they say well there are no instructions.



Learning transferred into the workplace and implementation of an achievable and practical return to work plan



The work environment has a significant influence and impact on performance



Consider function first, neurological occupational therapy before or as an adjunct to traditional talking therapy.

For those with **persistent and recurring challenges**, the scaffolding of neurological occupational therapy will be essential.

The risk if you do not adopt a curious approach is **long term work absence or further harm**.



...I chose the employer because they said they were inclusive. I know I have needs, my University helped me to understand them.

Before I took the job the employer sent me information about their EDI approach, the reality was different. How they said they would support me, and what they promised was not the reality.

I feel traumatised by the experience.





...I chose the employer because they said they were inclusive. I know I have needs, my University helped me to understand them.

Before I took the job the employer sent me information about their EDI approach, the reality was different. How they said they would support me, and what they promised was not the reality.

I feel traumatised by the experience.

I am not this person, my family are workers, I see myself as capable and have lots to offer but I don't know how to get back from this place.



Timely and appropriate support is what individuals, and the workplace need



...(It is) like being on a boat on a lake in the fog and someone is saying to you, look the shore is just over there.....but I keep thinking I can't see the shore and what if there are rocks in the way and so I am floating round in circles trying my best to find a way.



Return to work full time in same role with modifications.

She feels supported and valued.

Early appropriate intervention works.

An active mindset of kindness after curiosity leads to understanding ourselves and others better

DWP says it is 'shocked' by its own disability tribunal record

9 March 2020 572 Comments

74% felt they did not receive adequate treatment because a medical professional **did not believe** the severity of their symptoms.



35% have avoided seeking medical advice because they felt **embarrassed**.

I kept my MS secret for years – those of us with unseen disabilities shouldn't have to hide them

By being honest I hope the public will understand more about long-term neurological disabilities like MS



By Benjamin Cohen

Those with intellectual disability, mental health condition, or living with autism, more likely to face discrimination in workplace

Story by Claran Brennan • 9mo • 3 min read

Irish Examiner

+ Follow

1.1K followers

Stroke survivors face financial struggle and discrimination at work

New figures show that some stroke survivors have been forced to sell their home to pay for medical expenses.

By Enda Brady, news correspondent

sky news

Monday 12 August 2019 12:11 UK

Rates of Parkinson's disease are exploding.
A common chemical may be to blame

Adrienne Matei

GOV.UK

Home > Health and social care > Public health

Press release

New figures show larger proportion of strokes in the middle aged

92% of those living with a neurological disorder feel affected by stigma



Chronic Pain: 100% (80% very much)

Myalgic Encephalomyelitis: 99.6% (74% very much)

Migraine/headache: 96% (58% very much)

ADHD: 91% (49% very much)

Dystonia: 89% (31% very much)

Multiple Sclerosis: 87% (24% very much)

Epilepsy: 86% (27% very much)

Parkinson's Disease: 85% (34% very much)

Kindness and curiosity is a choice



What does your brain need to function normally?

Beyond the nutrients from the food you eat, beyond the oxygen you breathe, beyond the water you drink, there's something else equally as important: it needs other people.

Normal brain function depends on the social web around us. Our neurones require other people's neurones to thrive and survive.



David Eagleman
Neuroscientist



Thank you for the full case study
[Louise StoriesBehindTheCases MagazineStyle KrysalisLtd](#)

Please email questions to
jo@krysalisconsultancy.co.uk



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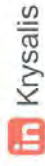
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